

Sanofi (SASY.PA)

Searching for reasons to be more positive, but pipeline rebuild will take time and valuation is insufficient; remain Neutral

SASY.PA	12m Price Target: €84.00	Price: €75.81	Upside: 10.8%
SNY	12m Price Target: \$49.00	Price: \$43.16	Upside: 13.5%

Following a period of R&D disappointments, despite a strong commercial engine, we see Sanofi as continuing to be in a period of transition. In our view, M&A/BD remains the key lever for the company to drive upside (with €15bn potential capacity for deals). However, given the need across the sector, competition for assets is likely to intensify. Looking across the portfolio today, duvakitug could be a key asset to help drive upside; however, data are not expected until 2H'28 and profits are shared with Teva. While some investors remain optimistic about the itepekimab opportunity, we believe this is blunted by the positive surprise from Astra's tozorakimab. Into 2Q'26 results, we are broadly in line with consensus and expect FY26 guidance to be reiterated. Given it will be the first set of results for the new CEO, we expect only high-level comments on strategy, with the focus being on initial impressions and key areas to be addressed (which likely include R&D productivity and M&A/BD given commercial execution has been strong, in our view). For us, a rebuild of the market's confidence in the Sanofi R&D engine (particularly given recent change in the head of R&D), as well as the post-Dupixent growth, will likely take time. We therefore remain Neutral and trim our target price to €84 (from €89).

Sanofi is due to report 2Q26 results on July 30 at 6.30am BST

- **2Q26 preview:** GSe is slightly below (c.-1%) consensus on sales and in line on Business operating income and Business EPS, and we expect guidance to be reiterated. Our Dupixent forecast is largely in line (+1%) with consensus, our Altuviiiio forecast 2% ahead, and Ayyakit 1% below. For 2Q26, we forecast a -1.7% FX impact on sales and a -2.2% impact on EPS. On the earnings call, we expect focus to be on the CEO's initial thoughts on the company/strategy, with possible implications for R&D pipeline

NEUTRAL

James Quigley
+44(20)7051-3800 | james.quigley@gs.com
Goldman Sachs International

Rajan Sharma
+44(20)7051-7995 | rajan.sharma@gs.com
Goldman Sachs International

Shyam Kotadia
+44(20)7051-6153 | shyam.kotadia@gs.com
Goldman Sachs International

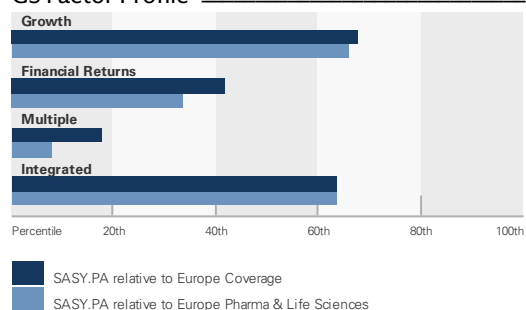
Key Data

Market cap: €91.2bn / \$104.4bn
Enterprise value: €103.9bn / \$119.3bn
3m ADTV: €190.4mn / \$221.2mn
France
Europe Pharma & Life Sciences
M&A Rank: 3
Leases incl. in net debt & EV?: Yes

GS Forecast

	12/24	12/25E	12/26E	12/27E
Revenue (€ mn) New	44,286.0	46,716.0	49,671.1	53,093.7
Revenue (€ mn) Old	44,286.0	46,716.0	49,523.3	52,914.6
EBIT (€ mn)	11,343.0	12,149.0	13,023.9	14,493.9
EPS (€) New	7.09	7.80	8.46	9.19
EPS (€) Old	7.09	7.80	8.46	9.19
P/E (X)	13.2	9.7	9.0	8.0
Dividend yield (%)	4.0	5.2	5.4	5.7
CROCI (%)	5.9	5.7	8.1	9.3
N debt/EBITDA (ex lease,X)	0.7	0.8	0.6	0.2
	3/25	6/25E	9/25E	12/25E
EPS (€)	1.79	1.59	2.90	1.52

GS Factor Profile



Source: Company data, Goldman Sachs Research estimates.
See disclosures for details.

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NEUTRAL

Sanofi (SASY.PA)

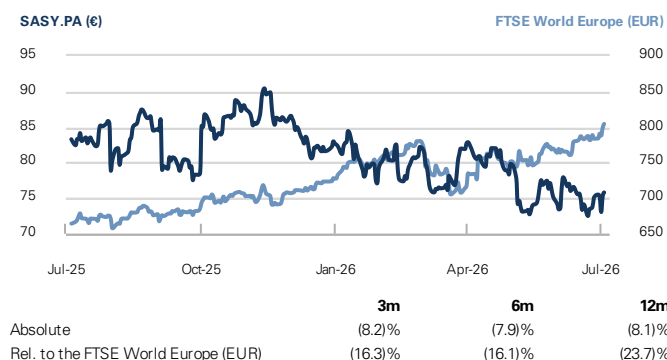
Rating since Mar 21, 2025

Ratios & Valuation

	12/24	12/25E	12/26E	12/27E
EV/sales (X)	2.9	2.3	2.0	1.8
EV/EBITDAR (X)	10.4	8.1	7.3	6.2
EV/EBITDA (excl. leases) (X)	10.2	8.0	7.1	6.0
EV/EBIT (X)	11.2	8.7	7.8	6.6
P/E (X)	13.2	9.7	9.0	8.0
Dividend yield (%)	4.0	5.2	5.4	5.7
EV/GCI (X)	0.8	0.7	0.6	0.6
CROCI (%)	5.9	5.7	8.1	9.3
ROIC (%)	10.5	11.7	13.3	14.8
ROA (%)	7.0	7.5	8.1	8.8
Days inventory outst, sales	78.7	76.7	70.1	61.8
Asset turnover (X)	3.8	4.0	4.1	4.0
Capex/D&A (%)	89.1	62.3	70.9	77.6
Net debt/equity (excl. leases) (%)	11.0	15.2	11.8	4.3
EBIT interest cover (X)	10.6	21.6	21.1	25.0
FCF cover of dividends (X)	1.2	1.4	2.1	2.1

Growth & Margins (%)

	12/24	12/25E	12/26E	12/27E
Total revenue growth	6.4	5.5	6.3	6.9
EBITDA growth	1.0	6.0	7.6	11.4
EBIT growth	1.5	7.1	7.2	11.3
Net inc. growth	3.0	40.5	2.4	27.6
EPS growth	(1.8)	9.9	8.0	12.3
DPS growth	5.7	4.3	5.1	5.0

Price Performance

Source: FactSet. Price as of 3 Jul 2026 close.

Balance Sheet (€ mn)

	12/24	12/25E	12/26E	12/27E
Cash & cash equivalents	7,442.0	7,657.0	10,454.6	12,929.2
Accounts receivable	7,677.0	8,410.0	9,075.7	9,729.0
Inventory	9,431.0	10,214.0	8,861.2	9,110.6
Other current assets	18,039.0	4,671.0	4,671.0	4,671.0
Total current assets	42,589.0	30,952.0	33,062.5	36,439.7
Net PP&E	11,601.0	11,511.0	12,738.1	13,998.2
Net intangibles	66,013.0	67,561.0	67,142.3	65,066.3
Total investments	316.0	3,259.0	3,310.0	3,310.0
Other long-term assets	12,280.0	13,522.0	13,522.0	13,522.0
Total assets	132,799.0	126,805.0	129,774.9	132,336.2
Accounts payable	7,551.0	7,361.0	7,553.0	7,797.1
Long-term debt	4,209.0	4,342.0	7,377.0	10,642.0
Short-term lease liabilities	262.0	272.0	272.0	272.0
Other current liabilities	17,141.0	16,370.0	16,547.5	16,534.9
Total current liabilities	29,163.0	28,345.0	31,749.5	35,246.1
Long-term debt	11,791.0	14,248.0	11,807.0	5,642.0
Long-term lease liabilities	1,645.0	1,467.0	1,467.0	1,467.0
Other long-term liabilities	12,343.0	11,035.0	10,928.9	11,430.6
Total long-term liabilities	25,779.0	26,750.0	24,202.9	18,539.6
Total liabilities	54,942.0	55,095.0	55,952.4	53,785.7
Preferred shares	--	--	--	--
Total common equity	77,507.0	71,376.0	73,488.5	78,216.6
Minority interest	350.0	334.0	334.0	334.0
Total liabilities & equity	132,799.0	126,805.0	129,774.9	132,336.2
Capital employed	93,857.0	90,300.0	93,006.5	94,834.6
Adj for unfunded pensions & GW	--	--	--	--

Cash Flow (€ mn)

	12/24	12/25E	12/26E	12/27E
Net income	5,561.0	7,813.0	7,999.3	10,204.5
D&A add-back	3,586.0	5,678.0	3,994.4	3,866.7
Minority interest add-back	58.0	38.0	--	--
Net (inc)/dec working capital	(141.0)	1,984.0	879.1	(658.5)
Other operating cash flow	18.0	(1,889.0)	237.4	489.1
Cash flow from operations	9,082.0	10,750.0	13,110.3	13,901.8
Capital expenditures	(3,195.0)	(3,538.0)	(2,832.8)	(3,000.8)
Acquisitions	(1,901.0)	(9,394.0)	(1,970.0)	(50.0)
Divestitures	--	0.0	--	--
Others	689.0	10,652.0	(167.0)	--
Cash flow from investing	(4,407.0)	(2,280.0)	(4,969.8)	(3,050.8)
Repayment of lease liabilities	(282.0)	(333.0)	--	--
Dividends paid (common & pref)	(4,742.0)	(4,810.0)	(4,937.8)	(5,176.5)
Inc/(dec) in debt	(624.0)	1,789.0	594.0	(2,900.0)
Other financing cash flows	(295.0)	(4,900.0)	(1,000.0)	(300.0)
Cash flow from financing	(5,943.0)	(8,254.0)	(5,343.8)	(8,376.5)
Total cash flow	(1,268.0)	216.0	2,796.6	2,474.5
Reinvestment rate (%)	34.6	40.4	23.2	20.6

Source: Company data, Goldman Sachs Research estimates.

Income Statement (€ mn)

	12/24	12/25E	12/26E	12/27E
Total revenue	44,286.0	46,716.0	49,671.1	53,093.7
Total operating expenses	(22,378.0)	(22,466.0)	(22,898.9)	(23,658.3)
R&D	(7,394.0)	(7,842.0)	(8,199.5)	(8,445.5)
Other operating inc./(exp.)	(3,171.0)	(4,259.0)	(5,548.8)	(6,496.0)
EBITDA	12,384.0	13,126.0	14,129.6	15,734.6
Depreciation & amortisation	(1,041.0)	(977.0)	(1,105.7)	(1,240.7)
EBIT	11,343.0	12,149.0	13,023.9	14,493.9
Net interest inc./(exp.)	(263.0)	(262.0)	(353.0)	(281.9)
Income/(loss) from associates	--	--	--	--
Profit/(loss) on disposals	--	--	--	--
Total other net	--	--	--	--
Pre-tax profit	11,080.0	11,887.0	12,670.9	14,212.0
Provision for taxes	(2,168.0)	(2,332.0)	(2,534.2)	(2,842.4)
Minority interest	--	--	--	--
Preferred dividends	--	--	--	--
Net inc. (pre-exceptionals)	8,912.0	9,555.0	10,136.7	11,369.6
Post-tax exceptionals	(3,351.0)	(1,742.0)	(2,137.4)	(1,165.0)
Net inc. (post-exceptionals)	5,561.0	7,813.0	7,999.3	10,204.5
EPS (basic, pre-except) (€)	7.12	7.83	8.46	9.50
EPS (basic, post-except) (€)	4.44	6.40	6.67	8.53
Wtd avg shares out. (basic) (mn)	1,251.4	1,220.4	1,198.5	1,196.6
Tax rate (%)	19.6	19.6	20.0	20.0
Common dividends declared	4,705.2	4,784.1	4,937.8	5,176.5
DPS (€)	3.76	3.92	4.12	4.33

prioritisation, and any reassessment of the capital allocation strategy, particularly in relation to business development and M&A given recent activity in the immunology space. We also expect a focus on Dupixent dynamics in 2026, with key considerations being any incremental updates on potential life-cycle management initiatives.

- **Challenges facing the company:** We believe Sanofi is facing a dual challenge of balancing near- to mid-term execution driven by Dupixent with long-term portfolio renewal. While extending the durability of the Dupixent franchise through lifecycle management initiatives remains a near-term priority, Sanofi also faces a need to restore investor confidence in its ability to generate the next wave of growth. This challenge is amplified by a series of R&D disappointments, including: mixed Phase III results for itepekimab (shares -5% on the day), lower-than-expected efficacy data for amlitelimab (shares -8% on the day), the Phase III failure and subsequent Complete Response Letter (CRL) for tolebrutinib in PPMS (shares -3% on the day) and the recent discontinuation of Phase III MOBILIZE trial of rilipribart in Chronic Inflammatory Demyelinating Polyneuropathy (CIDP), which have weighed on market confidence in the company's innovation engine and ability to drive revenue and earnings growth beyond Dupixent. Investors are likely to look for tangible evidence that Sanofi, under the new CEO and head of R&D, can improve R&D execution, advance a differentiated late-stage pipeline capable of supporting future commercial growth, and selectively utilize disciplined, value-accretive BD/M&A to address portfolio gaps. In our view, demonstrating consistent progress across these strategic priorities will be critical to establishing a robust post-Dupixent growth narrative, rebuilding confidence in the company's R&D capabilities, and supporting a recovery in investor sentiment and valuation.
- **What are we looking at to become more positive/negative:** The following would make us more positive on the stock: **(a)** Progress is made on Dupixent lifecycle management, particularly through an extension of market exclusivity beyond the currently anticipated 2033–34 biosimilar entry window and evidence that longer-acting formulations provide meaningful clinical benefits beyond dosing convenience. **(b)** Pipeline execution improves, supported by positive and differentiated data from key assets including lunsekimig (detailed Phase 2 results expected at European Respiratory society conference in Sep 2026), frexalimab (Phase 3 readouts in Relapsing Multiple Sclerosis (RMS) expected in 2H27) and duvakitug (Phase 3 data in Ulcerative Colitis (UC) and Crohn's disease (CD) are expected in 2H28 and 2H29 respectively). Other positive developments would be successful regulatory progress for efdoralprin alpha (2H26 US submission) and encouraging early-stage developments in immuno-oncology (PD-1/IL-15) and gene therapy. **(c)** Management executes disciplined, value-accretive M&A, strengthening Sanofi's growth outlook beyond Dupixent and demonstrating an ability to deploy capital effectively into attractive commercial or bolt-on clinical-stage opportunities. We would become more negative on the stock if: **(a)** Shares re-rate materially without fundamental support, such as improvements in earnings expectations, pipeline de-risking or meaningful commercial catalysts, increasing the risk of future disappointment. **(b)** Further pipeline setbacks emerge, particularly in late-stage

development, reducing future growth potential, increasing reliance on the existing commercial portfolio and raising the need for external innovation, licensing and M&A to sustain long-term growth.

- **12-month price target updated to €84 (previously €89).** Key underlying updates to our model include: **(i)** FX update; **(ii)** lowering of riliprubart PoS from 40% to 10% following the recent update on Phase 3 MOBILIZE study discontinuation; **(iii)** increased tolebrutinib PoS to 100% from 75% following EU approval in nrSPMS; **(iv)** incorporating the impact of the €2.3bn bond issue; and **(v)** we now model biosimilar entry/price pressure for Dupixent starting from 2033 and reduce our terminal growth rate from 0% to -2% given R&D disappointments, collectively leading to a -4% impact on our DCF estimates.

2Q26 preview

After updating for FX, company commentary, prescription trends, and recent bond issue, we are c.1% below consensus for 2Q26 net sales: our Dupixent forecast is 1% ahead of consensus, our Altuviiiio forecast 2% ahead, and our Ayvakit forecast 1% below consensus. For Business Operating Income and Business EPS we are in-line with consensus

Key factors to consider into the quarter:

- Dupixent: We forecast 18% CER sales growth in the quarter in the US, and 17%-18% CER growth ex-US. Aide memoire highlights commentary from 1Q26 earnings call on Dupixent facing a tougher comparison base in 2H26 from launch dynamics.
- Altuviiiio: We forecast €387mn of sales for 2Q26 and +36% CER growth globally in the quarter, driven by strong uptake as reflected in prescription trends.
- Ayvakit: We forecast €200mn of sales for 2Q26 and are -1% below consensus.
- FX: For 2Q26 we forecast a -1.7% impact on sales (aide memoire states -2% for the quarter) and a -2.2% impact on EPS (aide memoire states -3%). Our forecast full-year FX impact on sales/EPS is -1.5%/-1.9%.

Exhibit 1: 2Q26 preview table

€ mn and per share

Group (business basis)	2Q26		GSe vs.	
	Gse	Cons.	Cons.	€mn
Net Sales	10,678	10,823	-1.3%	(145)
Other revenues	701	755	-7.1%	(54)
COGS	(2,922)	(3,136)	-6.8%	214
Gross profit	8,458	8,442	0.2%	16
Margin, %	79.2%	78.0%		
R&D	(1,978)	(1,963)	0.7%	(15)
SG&A	(2,381)	(2,374)	0.3%	(7)
Other operating income/expense	(1,204)	(1,212)	-0.7%	8
Associate income	32	35	-8.9%	(3)
Minority interests	(1)	(3)	-66.7%	2
Business Operating Income	2,927	2,923	0.1%	4
Margin, %	27.4%	27.0%		
Financial Income/Expense	(92)	(106)	-13.2%	14
Pretax income	2,836	2,820	0.6%	16
Tax	(581)	(570)	2.0%	(11)
Tax Rate	-20.5%	-20.2%		
Business Net income	2,254	2,253	0.0%	1
Business EPS, Basic (€)	1.88	1.88	0.2%	0.00

Key Products	2Q26		GSe vs.	
	Actual	Cons.	Cons.	€mn
Total Net sales	10,678	10,823	-1%	(145)
Total Dupixent	4,452	4,413	1%	39
Total Pharma Launches	1,295	1,304	-1%	(9)
ALTUVIIIIO	387	380	2%	7
Sarclisa	156	169	-8%	(13)
Rezurock	147	142	3%	5
Nexvzyme	216	216	0%	0
Ayvakit	200	202	-1%	(2)
Xenpozyme	68	65	5%	3
Cablivi	70	72	-3%	(2)
TZIELD	25	25	-1%	(0)
Qfitlia	10	16	-38%	(6)
Wayrilz	18	17	3%	1
Total Other Products (Pharma)	2,962	2,998	-1%	(36)
Fabrazyme	263	263	0%	(0)
Cerezyme	169	168	1%	1
Alprolix	142	139	2%	3
Eloctate	55	54	2%	1
Myozyme	124	120	4%	4
Kevzara	136	144	-6%	(8)
Aldurazyme	70	69	1%	1
Cerdelga	81	81	0%	0
Jevtana	56	56	0%	0
Aubagio*	52	45	16%	7
Fasturtec*	44	42	6%	2
Toujeo, total	342	349	-2%	(7)
Praluent	152	147	4%	5
Plavix	227	222	2%	5
Thymoglobulin	126	126	0%	0
Lovenox	197	186	6%	11
Multaq	70	76	-8%	(6)
Mozobil*	8	7	16%	1
Lantus, total	388	399	-3%	(11)
Aprovel	95	99	-4%	(4)
Soliqua/iGlarLixi	67	68	-2%	(1)
Industrial Sales	96	138	-31%	(42)
Other pharma	777	863	-10%	(87)
Total Vaccines	1,193	1,245	-4%	(52)
Influenza Vaccines, total sales	101	135	-25%	(34)
Polio / Pertussis / Hib including Boosters	703	727	-3%	(24)
Beyfortus	87	83	5%	4
Meningitis / Travel and endemic vaccines	302	301	0%	1
Other vaccines	-	-	-	0

Vara consensus, last updated 1 July 2026

* Visible Alpha consensus, rest Vara consensus

Source: Vara Research, Visible Alpha Consensus Data, Goldman Sachs Global Investment Research

FY26 guidance: We expect Sanofi to reiterate its current CER guidance: high single-digit growth for sales and slightly faster than sales CER growth for Business EPS, with company commentary from 1Q26 indicating caution on the outlook for the remainder of the year as comparators become tougher, resulting in slower growth and limited revisions to FY26 company-compiled consensus post 1Q26. Overall, the mid-point of current guidance implies modest (c.1%) downside risk to FY26 consensus sales and Business EPS estimates.

Exhibit 2: FY26 guidance

	FY26 guidance at 4Q25	FY26 guidance at 1Q26	Gse:FY26 guidance at 2Q26	Midpoint of CER growth	Midpoint of FX	GSe FX	FY25 Actual	FY26 midpoint of guidance ex.buyback	Midpoint of guide including buyback	GSe	Vara Cons FY26	Midpoint vs cons (post buyback)	Implied midpoint vs. Consensus (using GSe FX (post buyback))
Sales	High single digit growth at CER	REITERATED: High single digit growth at CER	REITERATED: High single digit growth at CER	8.0%	-2.0%	-1.5%	43,626	46,174	46,174	46,960	46,693	-1.1%	-0.6%
EPS	Grow slightly faster than sales (before share buyback)	REITERATED: Grow slightly faster than sales (before share buyback)	REITERATED: Grow slightly faster than sales (before share buyback)	9.0%	-3.0%	-1.9%	7.83	8.28	8.43	8.71	8.54	-1.3%	-0.2%

Source: Vara Research, Company data, Goldman Sachs Global Investment Research

Key focus areas for the earnings call: (1) A potential strategy update following the recent leadership transition, with possible implications for R&D pipeline prioritisation, and any reassessment of the capital allocation strategy, particularly in relation to business development and M&A, given recent activity in the immunology space; **(2)** Dupixent dynamics in 2026, with key considerations being any incremental updates on potential life cycle management initiatives; **(3)** Altuviiiio and Ayvakit commercial dynamics and forward expectations; and **(4)** commentary on key pipeline readouts for the year – lunsekimig Phase 2 detailed AIRCULES data in asthma at ERS conference in Sep 2026 and amlitelimab's Phase 3 ESTUARY, including the outlook for amlitelimab approval.

What could make us more positive/negative on the stock

In the near-to-mid term, we see dual challenges for the new leadership: extending the durability of the Dupixent franchise, while simultaneously demonstrating a credible path towards replacing its sales and profitability either through delivery on internal pipeline or BD/M&A.

We see the following as the key developments which could make us more positive on the stock, and which could potentially help rebuild investor confidence in Sanofi's R&D engine:

1) Progress on Dupixent lifecycle management initiatives: Lifecycle management remains a key area of investor focus, particularly as the new CEO begins to lay out long-term strategic priorities. Greater clarity on the scope for Dupixent's period of market exclusivity to extend beyond current market expectations could represent meaningful valuation upside by delaying biosimilar entry (consensus currently expects 2033-34 biosimilar entry). In addition, evidence that longer-acting Dupixent formulations offer meaningful differentiation beyond dosing convenience, such as improvements in efficacy, safety or patient outcomes, could further enhance franchise durability in an increasingly competitive market.

2) Rebuild and delivery of the pipeline: Following a period marked by pipeline disappointments, sentiment around the pipeline is very low. Key drivers to help raise sentiment are: **(a)** near-term catalysts: detailed Phase 2 lunsekimig data in asthma, to be presented at the annual European Respiratory Society meeting in September 2026, a decision on the future development path of itepekimab in COPD, and Phase 3 frexalimab results in Relapsing Multiple Sclerosis (RMS) and Non-Relapsing Secondary Progressive MS (nrSPMS) (expected in 2H27, primary completion in May 2027); **(b)** mid-term value drivers: Phase 3 duvakitug data in Ulcerative Colitis (UC) and Crohn's disease (CD), expected in 2H28 and 2H29 respectively, represent a potentially differentiated opportunity in a fast-evolving and increasingly competitive IBD market; approval and launch of edoralprin alpha, a differentiated recombinant AAT therapy with potential regulatory clarity in 2H26 and longer-term upside from increased testing and earlier diagnosis of AATD-related COPD; and **(c)** early stage optionality: Sanofi's early stage pipeline is not reflected in our forecasts, but could provide upside over time. The PD-1×IL-15 asset offers high-risk, high-reward exposure to next-generation immuno-oncology, while the gene therapy platform could become a more meaningful growth driver if early clinical data in ophthalmology and rare diseases prove encouraging.

3) Value-accretive M&A: Disciplined, value-accretive M&A could help reinvigorate investor sentiment by strengthening confidence in Sanofi's ability to diversify growth ahead of Dupixent's eventual loss of exclusivity. Management has indicated capacity to deploy up to €15bn in 2026, and our analysis of prior deals suggests commercial-stage acquisitions offer the fastest path to earnings accretion, while clinical-stage bolt-ons provide the strongest long-term return potential. Successful capital deployment would represent an important step towards building the next growth platform beyond Dupixent.

The following could make us more negative on the stock:

1) Valuation re-rating without fundamental support: We could become more negative if Sanofi's shares were to materially outperform and trade at a premium without corresponding improvements in earnings expectations, pipeline de-risking or meaningful commercial catalysts. In our view, an expansion in valuation unsupported by fundamentals would leave limited scope for further upside while increasing downside risk if expectations are not met.

2) Further pipeline disappointments: Additional late-stage pipeline setbacks or weaker-than-expected clinical data would increase our caution, particularly given the importance of internal innovation to Sanofi's

medium-term growth outlook. Further clinical disappointments would reduce future revenue potential and increase reliance on the existing commercial portfolio to sustain earnings momentum, while making Sanofi increasingly reliant on external innovation and M&A/licensing to support long-term growth.

Dupixent: how much value remains to be unlocked through lifecycle management?

Dupixent remains the central pillar of the Sanofi investment story and continues to drive the majority of the company's earnings momentum. In 1Q26, Dupixent sales beat consensus by c.7%, driving 95% of the Group's top-line beat. While we expect Dupixent to remain the key growth driver, we anticipate its momentum to slow, as also noted by Sanofi in its 1Q26 earnings commentary, as the company enters a period of tougher comparators (note that in 2025, Dupixent received approvals in additional disease areas, including Chronic Spontaneous Urticaria (CSU - April-25 in the US) and Bullous Pemphigoid (BP - June-25 in the US), and advanced the commercial rollout in Chronic Obstructive Pulmonary Disease (COPD) across geographies).

Against this backdrop, lifecycle management remains a key area of investor focus, particularly as the new CEO begins to lay out long-term strategic priorities. Sanofi outlined a 'Defend, Extend, Innovate' strategy for Dupixent lifecycle management, with the company emphasizing a strong patent portfolio extending to 2045, and noting its intent to vigorously defend all patents. In our Sandoz initiation (see [here](#)), we note that average Biosimilars in the US launched c.1.5-2 years after the compound patent expiry (Sanofi recently highlighted that its analysis suggested US biosimilar launch 2-6 years post compound patent expiry).

- **Defend** - Pushing out the patent expiry beyond the market's base case for biosimilar entry in 2033-34 could drive upside to our DCF. Dupixent compound patents are seen by investors as expiring in 2031/32 - in the US, Dupixent compound patents are anticipated to expire in March 2031, with the European patent expiry is generally expected in September 2032, although methods of use patents extend into the early 2040s. Our discussions with investors suggest that the expectation in the market is for biosimilars to enter through 2033-34 as a base case. Currently, there are at least six biosimilars of dupilumab in development (Sandoz, Advanz/Alvotect, Formycon, Bio-Thera, Amneal Biosciences, Samsung Bioepis), and while biosimilar entry in the US could be pushed back by a few years, we would anticipate biosimilar entry in Europe shortly after patent expiration. Our analysis of the US/EU biosimilar markets (see our [Sandoz initiation](#)) highlights that biosimilars are generally launched in Europe in the year of patent expiry/LoE (or within 6-12 months), but delays in the US can be 1.5-2 years. Greater clarity around IP extension has been seen by some investors as a potential meaningful near-term catalyst - however, Sanofi noted there have been no patent challenges, and hence no settlements, and therefore there is no set time-frame to communicate to give more clarity to the market. We now model biosimilar entry/price pressure for Dupixent starting from 2033 and reduce our terminal growth rate from 0% to -2% given R&D disappointments, implying a -4% impact on our DCF estimates.
- **Extend** - Longer-acting formulations could extend the Dupixent franchise, offering longer treatment intervals combined with the trusted safety profile - we see this as only partially protecting the Dupixent franchise. At the 1Q26 results, Sanofi noted progress on a longer-acting Q4W Dupixent formulation, describing the approach as well-precedented, and enabled by higher concentrations or in combination with hyaluronidase. The company highlighted ongoing development of a higher-dose

Q4W regimen in asthma, noting that details on the development plan with partner Regeneron will be shared going forward. Currently, development is focused on the Q4W dosing interval, where there are already options available, and as such we see this as only partially protective of the franchise, particularly in ex-US markets.

- **Innovate** - Of note, Regeneron has also highlighted a number of assets not included in the Sanofi alliance (Long-acting IL13 & IL4 antibodies with optimised binding properties and long acting IL-4xIL-13 bispecific). While a longer-acting formulation of Dupixent could provide an additional lifecycle management lever, we view the commercial impact as less clear cut. In addition, the competitive landscape continues to evolve, with emerging agents such as ABBV's zumilokibart seeking to differentiate through less-frequent dosing schedules while delivering comparable efficacy. As a result, dosing convenience alone may not be sufficient to materially extend the franchise, unless accompanied by clear advantages in efficacy, safety or patient outcomes. The use of co-contracting could be a driver, however, with PBM reform it is unclear how this will remain a benefit in the future.

Overall, we see a dual challenge for management: extending the durability of the Dupixent franchise while simultaneously demonstrating a credible path towards replacing its sales and profitability, either through delivery of the internal pipeline or BD/M&A. Below we outline the key pipeline opportunities we see, and our view on a value-accretive M&A strategy. Progress on either front under new leadership could be a key driver of upside for the stock and help reinvigorate investor sentiment, we believe.

What are the key pipeline opportunities to restore confidence in Sanofi's R&D engine?

Following a period marked by pipeline disappointments we revisit what we consider to be the key pipeline opportunities, areas that could increasingly become areas of focus for Sanofi's new leadership. While execution risk remains, the pipeline offers a combination of near-term catalysts, mid-term value drivers, and earlier-stage optionality across immunology, neurology, oncology, rare disease, and gene therapy. Successful execution across even a subset of these programs could help reshape investor perception of the R&D engine and support a more diversified, long-term growth profile. Broadly we group these opportunities as:

Near-term catalysts

- Lunsekimig – Detailed Phase 2 results are to be presented at the annual European Respiratory Society meeting (Sep 2–9, 2026). In the AIRCULES phase 2b study, lunsekimig met its primary and key secondary endpoints, demonstrating a statistically significant and clinically meaningful reduction in exacerbations, and improvement in lung function as measured by pre-bronchodilator forced expiratory volume in one second (pre-BD FEV1). In the DUET Phase 2a study in Chronic Rhinosinusitis with Nasal Polyps (CRSwNP), lunsekimig met its primary endpoint of change in nasal polyp score from baseline and met its key secondary endpoints of change in patient reported nasal congestion/obstruction score and change in Lund-Mackay Computed Tomography (LMK-CT) score, vs. placebo at Week 24. Our bar for success for lunsekimig in asthma is TSLP-like control of exacerbations, with stronger data on FEV1 and SGRQ measures (Tezspire demonstrated a 0.29–0.44 rate ratio vs. placebo on exacerbation reduction, with a 0.13L improvement in FEV1 vs. placebo). In our model, we include peak unadjusted sales of €3bn at a 40% PoS.
- Frexalimab – Phase 3 readouts in Relapsing Multiple Sclerosis (RMS)(FREXALT) and Non-Relapsing Secondary Progressive MS (nrSPMS)(FREVIVA) (expected in 2H27 and 1H28, primary completion in May 2027 and March 2028 respectively), alongside broader expansion into kidney transplant rejection and renal disease, could establish CD40L inhibition as a differentiated immunology platform. *In our model, we include peak unadjusted sales of €3bn at a 40% PoS.*
- Itepekimab – Following mixed development history and recent positive validation of the mechanism from AstraZeneca's tozorakimab, management's decision on the future development path for itepekimab in Chronic Obstructive Pulmonary disease (COPD) remains an important near-term decision. However, with the breadth of the tozorakimab data vs. the mixed itepekimab data, we believe that tozorakimab is in a much stronger position. *In our model, we include peak unadjusted sales of €2.5bn across indications at a 20% PoS.*

Mid-term value drivers

- Duvakitug – Strong Phase 2b induction and maintenance data have positioned the asset as one of the leading TL1A programs in Inflammatory Bowel Disease (IBD), with additional upside from potential expansion into non-IBD inflammatory and fibrotic indications, where Sanofi hopes to be a fast follower. Phase 3 data from the

SUNSCAPE 1 and 2 and STARSCAPE 1 and 2 trials in Ulcerative Colitis (UC) and Crohn's disease (CD) are expected in 2H28 and 2H29 respectively. While the IBD market is large and rapidly evolving, duvakitug is likely to be third to market, potentially limiting any first-mover advantage. Furthermore, Sanofi is yet to outline a broader development or expansion strategy that could support differentiation across additional indications. *In our model, we include peak unadjusted sales of €2.3bn at a 60% PoS.*

- Efdoralprin alfa - A differentiated recombinant AAT therapy with potential regulatory clarity in 2H26 and longer-term upside from increased testing and earlier diagnosis of AATD-related COPD. *In our model, we include peak unadjusted sales of €1.2bn at an 80% PoS.*

Earlier-stage optionality (we currently do not include any sales in our model for the below assets given the early stage nature of these programmes).

- PD-1 IL-15 program - An emerging oncology asset seeking to combine checkpoint inhibition with targeted immune activation, potentially addressing historical limitations of IL-15-based therapies. At our Innovation Summit in March 2026, Sanofi pointed to PD-1xIL-15 bispecific approaches in solid tumors, describing the mechanism as biologically plausible and opening up new therapeutic avenues, although still at an early stage with initial data appearing promising in 3L mCRC.
- Gene therapy portfolio - Early stage programs in ophthalmology and neuromuscular disease provide longer-dated optionality and could become increasingly relevant under a renewed focus on next-generation therapeutic modalities.

Exhibit 3: Pipeline origin overview

Asset	Mechanism	Phase	Development Origin	Stage at Acquisition/In-licensing	Details
amtelimab	OX40L mAb	Phase 3	Acquisition	Phase 2a	Acquired via the \$1.4 billion acquisition of Kymab in 2021.
duvakitug	TL1A mAb	Phase 3	In-licensing / Collaboration	Phase 2b	In-licensed and co-developed through an exclusive collaboration with Teva Pharmaceuticals established in 2023.
ltepekimab	IL33 mAb	Phase 3	In-licensing / Collaboration	Preclinical/discovery	Co-developed with Regeneron Pharmaceuticals under their long-standing global collaboration agreement.
lunsekimig	IL13/TS1P Nanobody VHH	Phase 3	Platform acquisition	Preclinical/discovery	Developed using the proprietary Nanobody platform acquired through the \$4.8 billion purchase of Ablynx in 2018.
frexalimab	CD40L mAb	Phase 3	In-licensing	Preclinical/Phase 1	In-licensed from ImmuneNext in 2017.
elenestnib	D816V-mutated KIT inhibitor	Phase 3	Acquisition	Phase 2/3	Acquired via the \$9.1 billion acquisition of Blueprint Medicines in 2025.
venglustat	oral GCS inhibitor	Phase 3	Acquisition	Preclinical	Inherited through Sanofi's acquisition of Genzyme in 2011.
tolebrutinib	BTX inhibitor	Phase 3	Acquisition	Phase 2b	Acquired via the \$3.7 billion acquisition of Principia Biopharma in 2020.
niprubart	C15 mAb	Phase 3	Acquisition	Preclinical/Phase 1	Acquired via the \$11.6 billion acquisition of Bioverativ in 2018.
SPO087	vero cell	Phase 3	Internally Developed	-	-
SPO202	Z1 valent conjugate	Phase 3	In-licensing / Collaboration	Preclinical/Phase 1	Co-developed in partnership with SK Bioscience.
SPO218	vero cell	Phase 3	Internally Developed	-	-
balinatunfib (SAR441566)	oral TNFR1	Phase 2	Internally Developed	-	-
brivekimig	TNFαOX40L Nanobody VHH	Phase 2	Platform acquisition	Preclinical	Developed using the proprietary Nanobody platform acquired through the \$4.8 billion purchase of Ablynx in 2018.
SAR449028 (formerly BLU-808)	wild-type KIT inhibitor	Phase 2	Acquisition	Preclinical / Phase 1	Acquired via the \$9.1 billion acquisition of Blueprint Medicines in 2025.
SAR444336 (formerly THOR-909)	non-beta IL2 Synthorin	Phase 2	Acquisition	Preclinical	Acquired via the \$2.5 billion acquisition of Synthorx in 2019.
SAR445399	IL1R3 mAb	Phase 2	In-licensing / Collaboration	-	In-licensed from MAB Discovery
rovadictinib	JAK/ROCK inhibitor	Phase 2	In-licensing / Collaboration	Phase 2	In-licensed from Sino Biopharmaceutical / Chia Tai Tianqing (CTTQ) in March 2026 for \$135 million upfront and up to \$1.395 billion in milestones. It was cleared for Phase 2 in the U.S.
efdoralprin alfa (SAR447537)	AAT fusion protein	Phase 2	Acquisition	Phase 2	Acquired via the \$2.2bn acquisition of Inhibrx in May 2024
SAR445977	PO-1/IL15	Phase 2	Acquisition	Preclinical	Acquired via the \$1.9 billion acquisition of Kadmon in 2021
SAR402663	sFLT1 AAV gene therapy	Phase 2	Acquisition	Preclinical / Phase 1	Inherited through Sanofi's acquisition of Genzyme in 2011
SPO256	mRNA	Phase 2	Platform acquisition	Preclinical	Developed using the mRNA platform acquired through the \$3.2 billion purchase of Translate Bio in 2021.
SPO268	mRNA	Phase 2	Platform acquisition	Preclinical	Developed using the mRNA platform acquired through the \$3.2 billion purchase of Translate Bio in 2021.
SPO289	mRNA	Phase 2	Platform acquisition	Preclinical	Developed using the mRNA platform acquired through the \$3.2 billion purchase of Translate Bio in 2021.
SPO335	inactivated adjuvanted	Phase 2	Internally Developed	-	-
SAR446422	CD28xOX40 bispecific Ab	Phase 1	Internally Developed	-	-
SAR446959	NMFP13xADAMTSSxCAP Nanobody	Phase 1	Acquisition	Preclinical	Developed using the proprietary Nanobody platform acquired through the \$4.8 billion purchase of Ablynx in 2018
SAR448501	CD20 bispecific mAb	Phase 1	Acquisition	Phase 1	Acquired via the acquisition of Dren Bio's affiliate (Dren 0201)
SAR446597	B6xCTs AAV gene therapy	Phase 1	Internally Developed	-	-
SAR448951	TRIM2 agonist	Phase 1	Acquisition	Phase 1	Acquired via the \$470 million acquisition of Vigil Neuroscience in August 2025.
SAR446268	DMPK AAV gene therapy	Phase 1	Internally Developed	-	-
SAR445953	CEACAM5-Topo1 ADC	Phase 1	In-licensing / Collaboration	Preclinical	Co-developed in partnership with Seagen (now Pfizer)
SAR446523	GPRC5D mAb	Phase 1	Internally Developed	-	-
SPO287	Fluzone HD+Nuvaxovid/Flublok+Nuvaxovid	Phase 1	In-licensing / Collaboration	Preclinical / Phase 1	Developed under Sanofi's collaboration with Novavax (established in May 2024). It is a combination vaccine of Flublok (Sanofi) and Nuvaxovid (Novavax) for influenza and COVID-19.
SPO291	mRNA	Phase 1	Platform acquisition	Preclinical	Developed using the mRNA platform acquired through the \$3.2 billion purchase of Translate Bio in 2021
SPO269	mRNA	Phase 1	Platform acquisition	Preclinical	Developed using the mRNA platform acquired through the \$3.2 billion purchase of Translate Bio in 2021
SPO340	subunit	Phase 1	Acquisition	Phase 1	Acquired via the \$1.15 billion upfront acquisition of Vical in July 2025
SPO341	subunit	Phase 1	Acquisition	Preclinical / Phase 1	Acquired via the \$1.15 billion upfront acquisition of Vical in July 2025
SPO342	subunit adjuvanted	Phase 1	Acquisition	Phase 1/2	Acquired via the \$2.2 billion acquisition of Dynavax Technologies in Dec 2025

Source: Company data, Goldman Sachs Global Investment Research

Near-term catalysts

Frexalimab:

Phase 3 readouts for Frexalimab in both relapsing multiple sclerosis (RMS) and

non-relapsing secondary progressive MS (nrSPMS) are expected in 2027 and could represent an important inflection point for Sanofi's neurology franchise, following the disappointments for tolebrutinib. Frexalimab targets the CD40L pathway, a differentiated mechanism positioned upstream of both adaptive and innate immune activation. Mechanistically, CD40L inhibition modulates T-cell, B-cell, macrophage, microglial, and dendritic cell activity without causing broad lymphocyte depletion, potentially offering a different balance between efficacy and immune preservation relative to anti-CD20 therapies. We note that continuous B-cell suppression with CD20's was a concern of the KOL at our innovation clinic back in March 2026 (see [here](#)).

Data so far in MS: In the Phase 2 RMS study, frexalimab demonstrated robust MRI efficacy, with the high-dose arm showing an ~89% reduction in new gadolinium-enhancing T1 lesions at Week 12 versus placebo, while the low-dose arm showed ~79% reduction. Importantly, these effects appeared durable with longer follow-up, with sustained suppression of inflammatory MRI activity observed out to ~96 weeks in extension data. Findings also showed both doses of frexalimab provided significant reduction in new or enlarging T2 lesions, a secondary endpoint of the study. The program also generated encouraging biomarker data, with neurofilament light chain (NfL) reductions of ~41% in the high-dose arm by Week 48, suggesting potential impact on underlying neuroaxonal injury beyond acute inflammatory control. Safety was also viewed favourably, particularly given the absence of broad lymphocyte depletion, serious infection signals, or immunoglobulin suppression typically associated with chronic B-cell depletion strategies.

Commercial outlook and positioning in MS: Sanofi is additionally advancing frexalimab in nrSPMS, which may ultimately represent a more differentiated opportunity for the asset, given the role of chronic innate immune activation and microglial biology in progressive disease. However, the broader multiple sclerosis (MS) market remains highly challenging (see our MS deep dive [here](#)). Anti-CD20 therapies are deeply entrenched, and we expect the class to maintain ~65% market share given their consistently strong efficacy, long-term data sets, and physician familiarity. As such, the efficacy bar for new entrants is high, and so market expectations for frexalimab currently appear relatively modest in consensus estimates. The competitive landscape is also becoming increasingly crowded, with BTK inhibitors advancing across RMS and progressive MS alongside a growing number of early-stage CAR-T approaches. In our view, frexalimab's opportunity may ultimately depend less on displacing CD20 therapies broadly and more on establishing a differentiated profile in patients where long-term immune preservation, progression biology, or chronic treatment tolerability become increasingly important considerations.

Other indications: Beyond multiple sclerosis, Sanofi has expanded frexalimab development into kidney transplant rejection (Phase 2/3 FREXERA study, primary completion in Aug 2029) and focal segmental glomerulosclerosis (FSGS) (Phase 2a RESULT study, primary completion in Dec 2026). In transplantation, the CD40/CD40L axis plays a central role in T-cell activation, B-cell maturation, antibody production, and antigen-presenting cell signalling, making it one of the most biologically validated co-stimulatory pathways in graft rejection. Current standard immunosuppressive regimens, primarily calcineurin inhibitors such as tacrolimus, are effective but associated with significant long-term toxicities including nephrotoxicity, infection risk, and cardiovascular complications. CD40L inhibition therefore offers the potential for more

targeted immune modulation without broad immune depletion. Importantly, according to studies newer Fc-engineered antibodies such as frexalimab are designed to avoid the thromboembolic issues that historically limited earlier CD40L programs. In FSGS, CD40/CD40L signalling has been implicated in podocyte injury, inflammation, and fibrosis in FSGS, providing a rationale for targeting the pathway in progressive kidney disease. While still early, success in these renal indications could support the idea that CD40L modulation has applicability across multiple immune-mediated settings beyond neurology alone and could materially strengthen confidence in both the mechanism and Sanofi's broader innovation capabilities.

Our estimates: In our model, we include peak unadjusted sales of €3bn at a 40% PoS. In the event of positive Phase 3 results, de-risking our current 40% PoS would lead to 8% upside to our DCF-based valuation, with failure implying 5% downside, all else equal.

Itepekimab

Itepekimab, Sanofi's anti-IL-33 antibody developed with Regeneron, has had a mixed clinical trajectory that has weighed on investor sentiment toward the IL-33 class (beyond AstraZeneca's tozorakimab). In Phase 3 COPD (AERIFY program), itepekimab delivered a split outcome, meeting its primary endpoint in AERIFY-1 with a c.27% reduction in moderate-to-severe exacerbations, but failing in AERIFY-2, where lower-than-expected exacerbation rates (partly COVID-related) likely reduced statistical power. While safety remained broadly comparable to placebo and management emphasised the need for further data interrogation, the inconsistency raised questions on both trial design and the robustness of IL-33 inhibition in COPD.

By contrast, AstraZeneca's tozorakimab has reported positive Phase 3 results across three COPD trials, demonstrating consistent efficacy not only in the primary population (former smokers) but also in broader patient groups across smoking status, eosinophil levels and disease severity. Mechanistically, tozorakimab is designed to inhibit both canonical ST2-mediated signalling and alternative IL-33 pathways, potentially providing broader suppression of IL-33 biology than itepekimab (which primarily targets the IL-33/ST2 axis) and Roche's astegolimab (which targets the ST2 receptor and did not meet the primary endpoint of a statistically significant reduction in the AER at 52 weeks in the Phase 3 ARNASA study). While this validates IL-33 as a therapeutic target, we believe it also raises the competitive bar for itepekimab. Even if itepekimab progresses to market, tozorakimab's differentiated profile and likely broader label are likely to limit its commercial opportunity.

Sanofi has signalled continued commitment to the asset beyond COPD, with ongoing development in chronic rhinosinusitis (with and without nasal polyps), indicating a broader respiratory franchise approach. In our model, we include peak unadjusted sales of €2.5bn across indications at a 20% PoS. De-risking our current 20% PoS leads to 6% upside to our DCF-based valuation, with failure implying 1% downside, all else equal.

Mid-term catalysts

Duvakitug and the TL1A opportunity:

Duvakitug is emerging as an increasingly important asset within Sanofi's pipeline, with growing conviction around both its clinical profile and commercial potential. Duvakitug has demonstrated best-in class efficacy signals in Phase 2b IBD trials, including encouraging data in both induction and maintenance settings, reinforcing confidence in durability of response. The asset currently sits third in the class in terms of development timelines, behind Roche and the class leader Merck & Co.). Beyond inflammatory bowel disease (IBD), where partner Teva Pharmaceutical (covered by Matt Dellatore) sees a c.US\$3-5bn peak sales opportunity, the program is being actively expanded into additional indications. Notably, Teva on its 1Q26 earnings call, outlined three development buckets: T2, non-T2, and fibrotic diseases, and emphasised a disciplined, criteria-driven approach to indication prioritisation based on market size, mechanistic rationale, probability of regulatory success, and speed to market, with further updates expected through 2026. Overall, duvakitug appears to be transitioning from a relatively underappreciated asset to a potentially meaningful growth driver, with upside linked not only to IBD but also to broader lifecycle expansion opportunities. Below, we explore its potential and relevance within IBD, as well as the opportunity set beyond IBD.

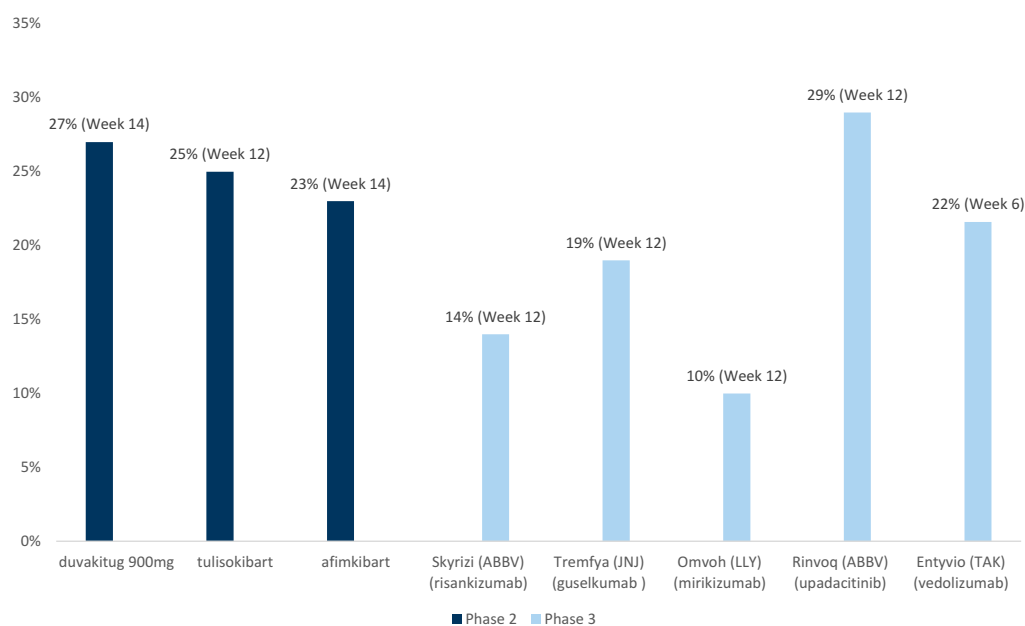
Mechanism of action and relevance in IBD: TL1A is a cytokine belonging to the tumor necrosis factor (TNF) superfamily. In healthy individuals, TL1A levels temporarily increase to help immune cells fight infections. However, in patients with IBD, TL1A is chronically overexpressed in the inflamed intestinal mucosa. Its mechanism of action is characterized by a dual role in driving both inflammation and fibrosis. Key drivers of clinical differentiation for this mechanism are:

- a) Dual-Action MoA (Inflammation + Fibrosis) and potential first-in-class anti fibrotic potential:** Unlike legacy biologics that solely target inflammatory cascades, TL1A blockade disrupts the TL1A-DR3 axis. This not only downregulates key pro-inflammatory cytokines (IFN- γ , TNF- α , IL-17) but also directly halts fibroblast activation and epithelial-to-mesenchymal transition. One of the most significant unmet needs in IBD (particularly in Crohn's disease) is the development of fibrostenosis (intestinal strictures). Currently, there are no approved medical therapies that directly target fibrosis in IBD, leaving surgical bowel resection as the only definitive treatment. Because TL1A directly stimulates fibroblasts, anti-TL1A antibodies have the unique potential to halt or even reverse collagen deposition and scarring, fundamentally altering the natural trajectory of the disease.
- b) Potential to raise the efficacy ceiling in refractory population.** Currently, existing biologics often fail to achieve long-term, deep remission. According to studies, approximately one-third of patients do not respond to initial treatment, and up to 40% lose response over time. TL1A inhibitors have demonstrated robust clinical remission and endoscopic healing rates in Phase 2 trials, offering a potentially effective option for patients with multi-biologic-refractory disease.
- c) Favourable Safety Profile:** Early clinical data suggest that TL1A inhibitors may not be as broadly immunosuppressive as traditional anti-TNF therapies. By acting more as an amplifier of specific inflammatory and fibrotic signals, rather than a broad immune suppressant, TL1A blockade may offer a safer long-term profile with a reduced risk of infections.

d) Enabling Precision Medicine: Genetic polymorphisms in the TNFSF15 gene, which encodes the TL1A protein, are strongly correlated with IBD risk and severity. This genetic link provides a rationale for using molecular biomarkers to identify specific patient subpopulations most likely to respond to TL1A blockade and thereby optimize trial probabilities of success and drive targeted commercial uptake.

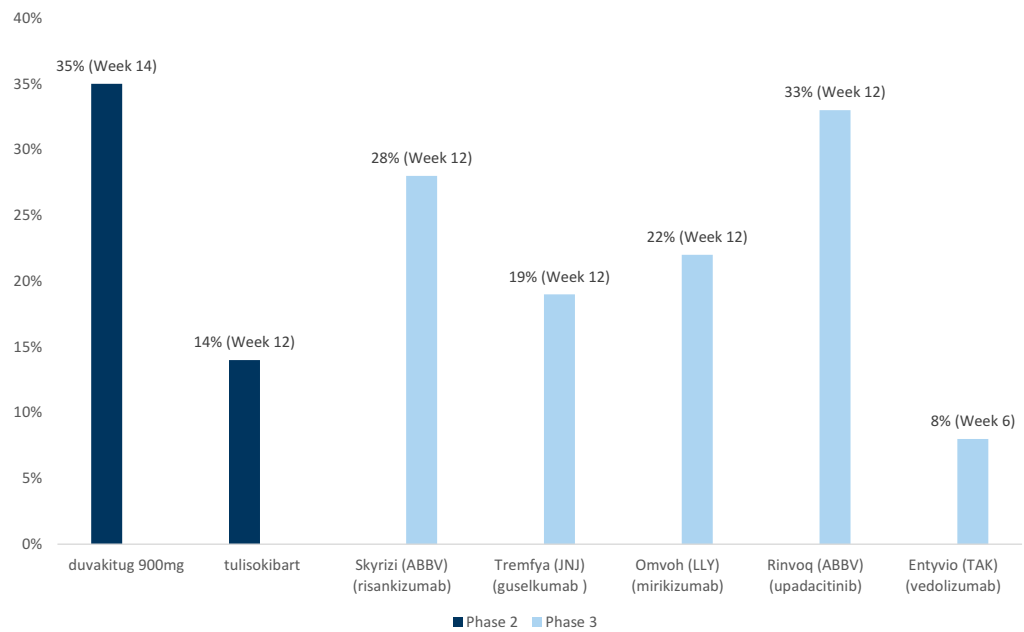
Data so far: Data from the Phase2b RELIEVE UCCD study demonstrated compelling efficacy at week 14 across both UC and CD. In UC, 36.2% (low-dose) and 47.8% (high-dose) of patients with UC treated with duvakitug achieved clinical remission compared to 20.45% on placebo - placebo-adjusted rates were 15.7% (low dose) and 27.4% (high dose) at week 14. Further higher clinical remission rates were observed for both doses of duvakitug versus placebo in both advanced therapy (AT)-experienced (22% and 29% placebo adjusted response in low- and high-dose respectively) and AT-naïve subgroups of patients (12% and 26% placebo-adjusted response in low- and high-dose respectively). In patients with CD, 26.1% (low-dose) and 47.8% (high-dose) treated with duvakitug achieved endoscopic response compared to 13.0% on placebo - placebo-adjusted rates were 13.0% (low dose) and 34.8% (high dose) at week 14. Higher endoscopic response rates were observed for both doses of duvakitug versus placebo in both AT-experienced (7% and 44% placebo adjusted response in low- and high-dose respectively) and -naïve subgroups of patients (22% and 29% placebo adjusted response in low- and high-dose respectively). Maintenance data indicate a highly differentiated profile for duvakitug, with rates of clinical remission and endoscopic response of 58% and 55% in UC and CD, respectively, at the 900mg Q4W dose alongside a clean safety profile.

Exhibit 4: Ulcerative Colitis induction data



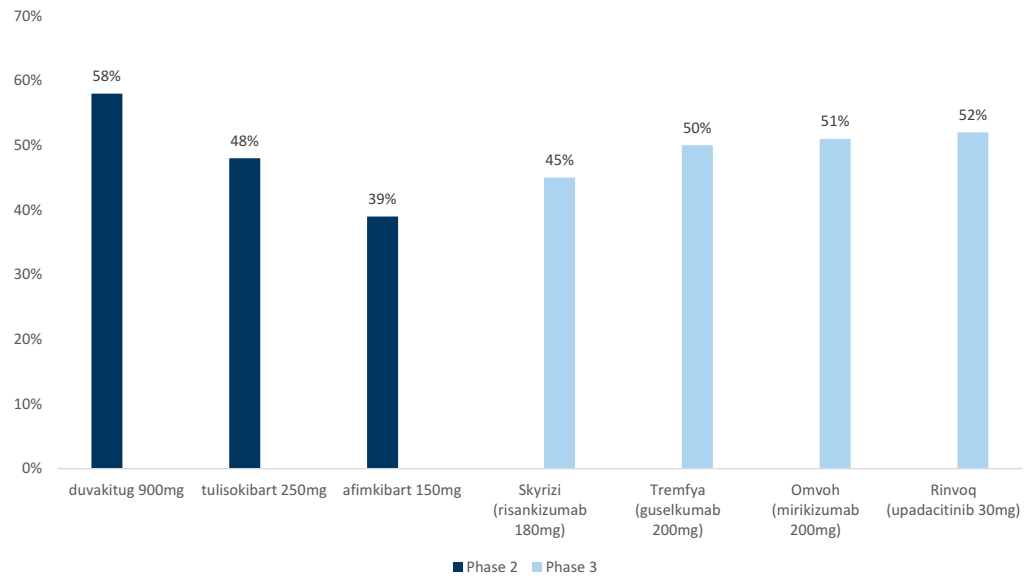
Source: Company data, Data compiled by Goldman Sachs Global Investment Research

Exhibit 5: Crohn’s disease induction data

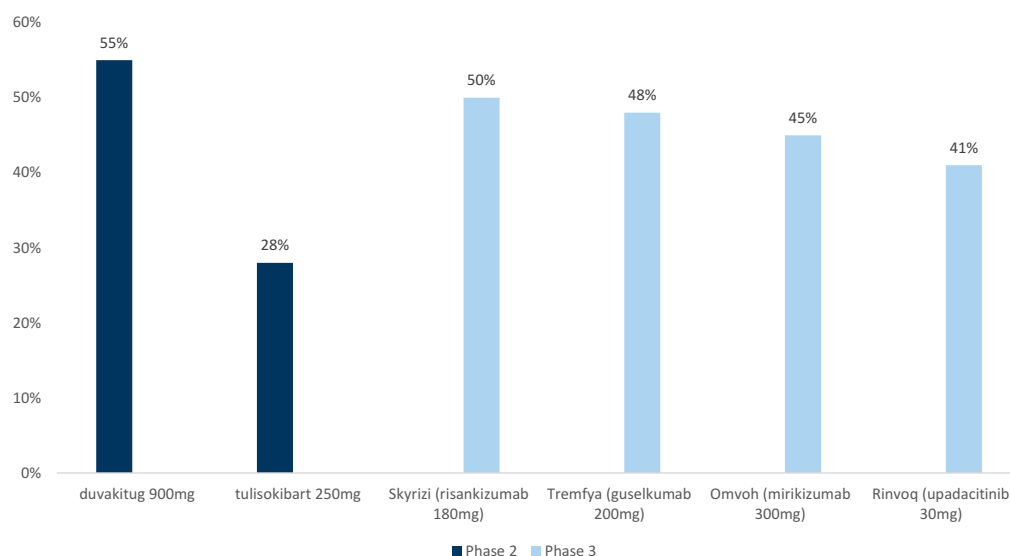


Source: Company data, Data compiled by Goldman Sachs Global Investment Research

Exhibit 6: Ulcerative Colitis maintenance data



Source: Company data, Data compiled by Goldman Sachs Global Investment Research

Exhibit 7: Crohn's disease maintenance data

Source: Company data, Data compiled by Goldman Sachs Global Investment Research

Competitor data: In our view, within the TL1A landscape, the two most advanced competitor assets to duvakitug are Roche's afimkibart and Merck & Co.'s tulisokibart, both of which have helped validate the mechanism but also frame the competitive debate around differentiation and timing. Across Phase 2 datasets, duvakitug has generated compelling efficacy signals in ulcerative colitis and a potentially best-in-class profile in Crohn's disease. By comparison, afimkibart demonstrated solid efficacy but with more moderate placebo-adjusted responses, while tulisokibart showed particularly strong headline efficacy that was partly aided by an unusually low placebo response in its Phase 2 studies, amplifying placebo-adjusted response rates. As a result, one of the key debates for the class is whether tulisokibart can reproduce these placebo-adjusted benefits in larger Phase 3 programs, where placebo response rates are typically higher and more variable. This becomes particularly relevant given Merck's recent update (see release [here](#)) that its Phase 3 ATLAS-UC trial met all primary and key secondary endpoints, although detailed efficacy and safety data are yet to be disclosed. If the full dataset supports rapid regulatory filings and a competitive efficacy profile, tulisokibart could narrow or eliminate duvakitug's potential first-in-class advantage. Conversely, if the placebo-adjusted efficacy appears more modest once detailed results are presented, attention may increasingly shift toward molecule-specific differentiation, including duvakitug's durability, maintenance profile and broader positioning potential across inflammatory and fibrotic indications.

Exhibit 8: Summary of TL1A data

Company Trial Phase Indication No. of participants Arms Dosing Primary endpoint: UC: Clinical remission CD: endoscopic response Subgroup response Biomarker positive Key secondary endpoint: UC: Endoscopic improvement CD: Clinical remission Biomarker positive Safety: AE SAE Aes leading to discontinuation Infection	Duvakitug				Tulisokibart		Afimkibart	
	SNY/Teva				Merck		Roche	
	RELIEVE-UCCD		RELIEVE-UCCD-LTE		ARTEMIS-UC	APOLLO-CD	TUSCANY-2	
	2				2b	2a	2b Induction	2b Maintenance
	UC	CD	UC	CD	UC	CD		
No. of participants	144	142			135	55		
Arms	450mg 900mg Placebo		450mg 900mg Placebo		500mg Placebo	500mg	50mg 150mg 450mg Placebo	50mg 150mg 450mg
Dosing	Q2W	Q2W			Q4W	Q4W	Q4W	Q4W
Primary endpoint: UC: Clinical remission CD: endoscopic response	Week 14 450mg: 36% (16%) 900mg: 48% (28%) Placebo: 20%	Week 14 450mg: 26% (13%) 900mg: 48% (35%) Placebo: 13%	Week 44 450mg: 47% 900mg: 58%	Week 44 450mg: 41% 900mg: 55%	Week 12: 500mg: 27% Placebo: 2%	Week 12: 500mg: 26%	Week 14: 50mg: 30% (18%) 150mg: 35% (23%) 450mg: 32% (20%) Placebo: 12%	Week 56: 50mg: 31% 150mg: 39% 450mg: 36%
Subgroup response	AT-experienced: 450mg: 29% (22%) 900mg: 36% (29%) AT-naïve: 450mg: 39% (12%) 900mg: 53% (26%)	AT-experienced: 450mg: 11% (7%) 900mg: 48% (44%) AT-naïve: 450mg: 47% (25%) 900mg: 47% (25%)	-	-	AT-experienced: 500mg: 22% (22%) AT-naïve: 500mg: 30.6% (27.6%)		-	-
Biomarker positive			-	-	Week 12: 500mg: 32% Placebo: 11%		-	-
Key secondary endpoint: UC: Endoscopic improvement CD: Clinical remission	Week 14 450mg: 45% 900mg: 50% Placebo: 23%	Week 14 450mg: 50% 900mg: 54% Placebo: 41%	-	-	Week 12: 500mg: 37% Placebo: 6%	Week 12: 500mg: 49%	Week 14: 50mg: 40% (21%) 150mg: 38% (19%) 450mg: 41% (22%) Placebo: 19%	Week 56: 50mg: 38% 150mg: 39% 450mg: 50%
Biomarker positive			-	-	Week 12: 500mg: 37% Placebo: 19%			
Safety:								
AE		450mg: 67% 900mg: 43% Placebo: 48%	-	-	46% vs 43%	78%	Week 14: 50mg: 34% 150mg: 45% 450mg: 53% Placebo: 56%	Week 56: 50mg: 61% 150mg: 50% 450mg: 66%
SAE			-	-			Week 14: 50mg: 6% 150mg: 0% 450mg: 3% Placebo: 9%	Week 56: 50mg: 9% 150mg: 0% 450mg: 14%
Aes leading to discontinuation		450mg: 9% 900mg: 2% Placebo: 2%	-	-	1% vs 3%	4%	Week 14: 50mg: 2% 150mg: 2% 450mg: 1% Placebo: 7%	Week 56: 50mg: 7% 150mg: 0% 450mg: 3%
Infection			-	-	17.8% vs 18.2%			

Source: Company data, clinicaltrials.gov

Our estimates: In our model, we include peak unadjusted sales of €2.3bn at a 60% PoS. In the event of positive Phase 3 results, de-risking our current 60% PoS leads to 2% upside to our DCF-based valuation, with failure implying 4% downside, all else equal. We do not include any significant sales beyond IBD for duvakitug and currently assume that the third to market classification holds back sales vs. the best in class disease data (as we await confirmation of the bic profile in Phase 3 trials).

Broader TL1A opportunity: The opportunity for TL1A inhibition extends well beyond IBD, and represents a “pipeline-in-a-pill” opportunity with relevance across immune-mediated and fibrotic conditions. TL1A (TNFSF15) sits upstream in the inflammatory cascade, modulating both adaptive and innate immune responses, and has been implicated in tissue inflammation as well as fibrosis, supporting a multi-indication expansion strategy, as also indicated by Sanofi at the Innovation Summit we hosted in March 2026 (see [here](#))

- T2 (Type 2 inflammatory) indications: TL1A plays a role in amplifying Th2-driven pathways, positioning it as a potential therapeutic target in diseases traditionally dominated by cytokines such as IL-4, IL-5, and IL-13. This includes conditions like asthma, atopic dermatitis, and chronic rhinosinusitis with nasal polyps. While these markets are already competitive, TL1A inhibition could offer differentiation through upstream pathway modulation and potential effects on tissue remodelling, which remains an unmet need in subsets of patients.

- **Non-T2 indications:** Beyond classical Th2 biology, TL1A is also involved in Th1 and Th17 pathways, broadening its relevance to diseases such as psoriasis, psoriatic arthritis, rheumatoid arthritis and other autoimmune conditions. This creates an opportunity to position TL1A inhibitors in indications where multiple immune axes intersect, potentially enabling efficacy across heterogeneous patient populations that are less well served by more pathway-specific biologics.
- **Fibrotic indications:** In our view the most differentiated opportunity potentially lies in fibrosis, where TL1A has been linked to fibroblast activation and extracellular matrix deposition. This includes indications such as pulmonary fibrosis, systemic sclerosis, MASH and other organ-specific fibrotic diseases, where there remains a high unmet need and limited disease-modifying therapies. The dual anti-inflammatory and anti-fibrotic potential of TL1A inhibition could be particularly compelling here, especially if supported by clinical data demonstrating impact on disease progression rather than just symptom control.

The approaches of Merck & Co. and Roche highlight the breadth of the TL1A opportunity beyond IBD. Merck is actively expanding tulisokibart into non-T2 and fibrotic indications, including rheumatoid arthritis, axial spondyloarthritis, hidradenitis suppurativa and systemic sclerosis-associated ILD and Roche is pursuing a complementary strategy spanning T2 and fibrotic diseases such as atopic dermatitis and MASH for afimkibart. While IBD remains the near-term anchor for Sanofi/Teva's duvakitug, the longer-term value will likely be driven by expansion into less-crowded, high unmet need indications, where Sanofi expects to be a fast follower.

Optionality in IBD: Sanofi's exposure to IBD extends beyond just duvakitug through two other mechanistically distinct assets: brivekimig (TNF×OX40L) and balinatunfib (oral TNF inhibitor). Collectively they provide Sanofi with multiple opportunities to participate in a growing immunology market. Importantly, the assets target different aspects of IBD biology and treatment paradigms - TL1A inhibition through modulation of inflammation and fibrosis, dual TNF/OX40L blockade through simultaneous targeting of inflammatory and T-cell co-stimulatory pathways, and an oral TNF inhibitor that could potentially deliver biologic-like efficacy with the convenience of oral administration. Among the three, duvakitug currently appears the most advanced and differentiated opportunity given the strength of its Phase 2b induction and maintenance data and the broader lifecycle potential of TL1A biology beyond IBD. Brivekimig offers exposure to a novel next-generation TNF-based approach that may potentially improve upon conventional TNF inhibition through OX40L modulation, while balinatunfib addresses a longstanding objective: translating the efficacy of TNF blockade into a convenient oral therapy. In a market increasingly characterised by intense competition across mechanisms, Sanofi has built a portfolio with multiple differentiated shots on goal across distinct therapeutic approaches, providing several potential avenues to establish a meaningful presence in the evolving IBD market.

Exhibit 9: Key pipeline assets in IBD

Drug	Indication	Company	Phase	Mechanism	Route of administration	Status/Data readout timing*
Tulisokibart (MK-7240)	UC/CD	Merck & Co	Phase 3	TL1A	Injectable biologic	2H26
Duvakitug	UC/CD	Sanofi/Teva Pharmaceutical	Phase 3	TL1A	Injectable biologic	2028/29
Ivarmacitinib	UC	Jiangsu Hengrui Pharmaceuticals/Arcutis Biotherapeutics	Phase 3	JAK1	Oral small molecule	-
Afimkibart	UC/CD	Roche	Phase 3	TNFS15	Injectable biologic	UC: 2Q27, CD:4Q28
Obefazimod	UC/CD	Abivax S.A.	Phase 3/Phase2b	miR-124	Oral small molecule	Jun-26
JNJ-4804	UC/CD	Johnson & Johnson	Phase 2b	IL-23 +TNF-alpha	Injectable biologic	Phase 3 initiated post Phase 2b results on May 5 2026
Zotemtegrast	UC/CD	Eli Lilly and Company	Phase 2b	α4β7 inhibitor	Oral small molecule	2030
Icotrokinra	UC	Johnson & Johnson/Protagonist Therapeutics	Phase 2b	IL-23	Oral biologic	2028
Eltrekibart	UC	Eli Lilly and Company	Phase 2	CXCR-2/IL-8	Injectable biologic	2028
Zasocitinib (TAK-279)	UC/CD	Takeda Pharmaceutical	Phase 2	JAK/STAT/TYK2	Oral small molecule	Phase 2b readout in 2026
ALT8-268	UC	AltruBio	Phase 2	PSGL-1	Injectable biologic	2026
Emvitegrast (GS-1427)	UC	Gilead Sciences	Phase 2	α4β7 inhibitor	Oral small molecule	2026
RG6730/PF-07261271	UC	Roche/Pfizer	Phase 2	p40xTL1A bispecific	Biologic	2029+
SPY001/SPY002	UC	Spyre Therapeutics	Phase 2	α4β7 inhibitor	Injectable biologic	2026/27
Tilpisertib fosmecarbil	UC	Gilead Sciences	Phase 2	MAP3K8	Oral small molecule	1H27
Brivekimig	UC/CD	Sanofi	Phase 2	TNF-alpha/OX40L	Injectable biologic	1H27
Balinatunfib (SAR441566)	UC/CD	Sanofi	Phase 2	Oral TNF R1	Oral small molecule	2H27
ABBV-8736	CD	AbbVie	Phase 2	TREM-1	Injectable biologic	2027/28
Litfulo (retlectitinib)	UC/CD	Pfizer Inc.	Phase 2	JAK/STAT/TYK	Oral small molecule	-
AZD-7798	CD	AstraZeneca	Phase 2	CCR9	Injectable biologic	2026
AGMB-129	CD	AgomAb Therapeutics NV	Phase 2	ALK5	Oral small molecule	2026
Skyrtzi +ABBV-382	CD	AbbVie	Phase 2	IL-23 + anti-a4b7	Injectable biologic	2027/28
Lusvertikimab	UC	OSE Immunotherapeutics	Phase 2	CD127	Injectable biologic	Likely initiation of Phase 2b/3 in UC in 2H27

* Status/readout timing based on company data or GSe based on primary completion

Source: Company data, Data compiled by Goldman Sachs Global Investment Research

Efdoralprin alpha

Efdoralprin alfa is an emerging asset within the rare disease pipeline, targeting alpha-1 antitrypsin deficiency (AATD) related emphysema. Mechanistically, it is a recombinant AAT-Fc fusion designed to restore functional AAT levels while enabling extended dosing intervals, addressing key limitations of plasma-derived augmentation therapies. The Phase 2 ElevAATe study delivered a positive dataset, demonstrating superiority in maintaining AAT levels within the normal range versus standard of care and supporting less frequent dosing (Q3W/Q4W), underpinning a differentiated profile on both efficacy and convenience. In the US, Sanofi expects to determine in 2H26 whether the existing dataset could support a filing, suggesting potential for an accelerated path. In Europe, however, a Phase 3 outcomes study is likely required, anchoring a base case of 2028 readout and 2028/29 launch for broader approval.

Treatment landscape in AATD-related emphysema: The current treatment landscape for AATD-related emphysema is dominated by four approved plasma-derived augmentation therapies- Prolastin-C (Grifols), Glassia (Takeda), Zemaira (CSL) and Aralast NP (Takeda) which have established alpha-1 antitrypsin replacement as the standard of care. These products are purified from human plasma and aim to restore circulating AAT levels sufficiently to inhibit neutrophil elastase-mediated lung damage. However, plasma-derived therapies are associated with several inherent limitations, including dependence on plasma collection and fractionation infrastructure, variability in supply, and the need for lifelong weekly intravenous infusions to maintain protective AAT concentrations. Efdoralprin alfa is designed as a recombinant AAT-Fc fusion protein that could potentially address several of these constraints. By leveraging Fc-mediated half-life extension, the asset has demonstrated the ability to maintain functional AAT levels within the normal range with Q3W/Q4W dosing in Phase 2 studies, representing a meaningful reduction in treatment burden versus current augmentation therapies. In addition, recombinant manufacturing removes reliance on plasma supply, potentially improving scalability and supply security. As such, if efdoralprin alfa is ultimately able to secure US approval based on the existing dataset, or if a future Phase 3 study establishes a sufficiently compelling clinical profile to support broader regulatory approval, the asset has the potential to meaningfully disrupt a plasma-derived AAT augmentation market.

Another key angle highlighted by management is the opportunity in premature COPD associated with AATD, which remains significantly under-diagnosed and under-tested today. Current clinical practice often identifies patients only once disease is advanced, limiting the impact of augmentation therapies. Sanofi has pointed to the lack of systematic screening and early testing for AATD in COPD populations as a major gap, suggesting that broader adoption of testing could materially expand the diagnosed patient pool. In this context, efdoralprin alfa could play a role not only as a treatment innovation but also as a catalyst for earlier intervention paradigms, where restoring AAT levels before significant lung damage occurs may have greater disease-modifying potential. If testing rates improve and earlier-stage patients are captured, this could represent a meaningful expansion of the addressable market beyond the traditional severe AATD population. Looking ahead, the trajectory of efdoralprin alfa will likely be defined not just by clinical outcomes, but by how effectively the field shifts toward earlier diagnosis and treatment. Greater uptake of AATD testing in COPD, particularly in patients with early or atypical disease, could meaningfully increase patient identification, positioning the asset to benefit from both improved clinical management

and a structurally expanding market.

Our estimates: In our model, we include peak unadjusted sales of €1.2bn at an 80% PoS. De-risking our current 80% PoS leads to 1% upside to our DCF-based valuation, with failure implying 5% downside, all else equal.

Early stage pipeline:

SAR445877 (PD-1 IL-15)

Mechanism: PD-1/IL-15 fusion proteins are designed to address a key limitation of checkpoint inhibitors by combining immune reactivation with immune amplification in a single construct. PD-1 blockade removes inhibitory signalling on exhausted T cells, while IL-15 drives proliferation and activation of CD8+ T cells and NK cells. By co-localising these effects, the approach aims to both reinvigorate and expand anti-tumour immunity directly within the tumour microenvironment, particularly in checkpoint-refractory or “cold” tumours. Critically, linking IL-15 to a PD-1 antibody enables more targeted cytokine delivery, which could improve the efficacy–toxicity balance relative to earlier systemic IL-15 approaches. At our Innovation Summit in March 2026, management described the mechanism as biologically plausible and opening up new therapeutic avenues, although still at an early stage with initial data appearing promising in 3L mCRC.

Data so far: Sanofi’s PD-1/IL-15 asset (SAR445877) remains in early clinical development, with initial Phase 1/2 data (ASCO 2025) suggesting a more encouraging profile than prior attempts in the class (Pfizer’s discontinued PD-1/IL-15 fusion, PF-07209960). While the dataset is small and early, it provides an initial signal that Sanofi may be achieving a more favourable balance between immune activation and tolerability, historically a key challenge for IL-15–based therapies.

Exhibit 10: PD-1/IL-15 Early stage data

	Sanofi: SAR445877 (PD-1/IL-15)	Pfizer: PF-07209960 (PD-1/IL-15)
Development Status	Active (Early clinical development Phase 1/2)	Discontinued
Trial Phase / Data Source	Phase 1/2 (ASCO 2025)	Phase 1
Patient Population	49 (checkpoint-refractory and 3 lines of prior therapy)	37 (CRC=26, NSCLC=1, OvCa=4, RCC=6), 54% had ≥4 prior lines of anticancer drug therapy
Objective Response Rate (ORR)	~14% (7 confirmed partial responses)	6.9% (2 patients had confirmed partial response)
Duration of Response (DoR)	> 1 year	3-9.5 months
Immunogenicity (ADA Rate)	~73%, did not correlate with clinical benefit or toxicity	~94%, of which 64% was treatment induced and 30% was treatment boosted
Safety & Tolerability	6 patients discontinued treatment due to any TEAE	All patients discontinued treatment. 76% had at least 1 TRAE

Source: Company data, Goldman Sachs Global Investment Research

Implications for Sanofi’s oncology franchise: SAR445877 represents a high-risk, high-reward opportunity within immuno-oncology, with potential exposure to large solid tumour markets, particularly in checkpoint-refractory settings. Relative to prior IL-15 approaches, early data suggest Sanofi’s SAR445877 may have potential to mitigate some of the key historical limitations. However, the program remains early and unproven, and the key inflection point will likely be whether these signals translate into reproducible, durable responses in larger Phase 2/3 trials without a step-up in toxicity or immunogenicity.

Strategically, the asset provides Sanofi with optionality in next-generation immuno-oncology, but at this stage it can be best viewed as a longer-dated pipeline lever rather than a near-term growth driver, with meaningful value contingent on continued clinical de-risking.

Gene therapy

Sanofi’s gene therapy portfolio remains early stage but provides longer-dated optionality across ophthalmology and neuromuscular disease, with the potential to complement its core franchises if clinical proof-of-concept is established. The company

is advancing multiple AAV-based programs, including: **(i)** SAR446597 (Bb C1s AAV) in Phase 1 for geographic atrophy (GA); **(ii)** SAR402663 (sFLT01 AAV) in Phase 2 for wet AMD; and **(iii)** SAR446268 (DMPK AAV) in Phase 1 for myotonic dystrophy type 1 (DM1). All three assets are built around the premise of one-time, durable expression of therapeutic proteins, aiming to shift treatment paradigms away from chronic dosing.

In ophthalmology, SAR446597 (Bb C1s AAV) is being evaluated in Phase 1 for geographic atrophy (GA) secondary to dry age-related macular degeneration. The program targets complement pathway dysregulation, a validated mechanism in GA, with the aim of providing sustained intraocular inhibition via a single administration, potentially improving on the burden and variability associated with frequent injections. SAR402663 (sFLT01 AAV) is in Phase 2 for wet AMD, where it seeks to deliver continuous anti-VEGF activity through gene expression, potentially reducing or eliminating the need for repeated anti-VEGF injections. Given the large and established anti-VEGF market, a durable gene therapy approach could represent a meaningful shift in treatment paradigm, particularly in patients requiring frequent dosing.

Among late-stage retinal gene therapy assets in wet AMD, the most advanced programs—AbbVie/REGENXBIO's ABBV-RGX-314, 4D Molecular Therapeutics's 4D-150, and Adverum Biotechnologies's Ixo-vec (ADVM-022) have demonstrated encouraging proof-of-concept around durable anti-VEGF expression and substantial reduction in injection burden. ABBV-RGX-314 has shown sustained intraocular anti-VEGF expression with preservation of visual acuity and meaningful reductions in rescue injections, supporting progression into multiple Phase 3 studies. Ixo-vec demonstrated c.80% reductions in annualized anti-VEGF injection burden in the OPTIC study while maintaining stable BCVA and retinal anatomy, although inflammation management remains an important focus for the program. 4D-150 has generated encouraging Phase 1/2 data with durable efficacy signals and a potentially differentiated profile through dual VEGF-A and VEGF-C inhibition. Overall, these programs suggest retinal gene therapy may increasingly approach the efficacy of current anti-VEGF standards while offering a potentially significant reduction in treatment burden through one-time administration. However, despite these encouraging datasets, we believe the key debate is likely to shift from whether gene therapy can reduce injections to whether it can achieve a sufficiently favourable long-term risk-benefit profile relative to increasingly durable injectable anti-VEGF therapies. In our view, major concerns remain around intraocular inflammation, controllability of transgene expression, long-term retinal safety, and physician willingness to adopt irreversible treatment approaches in a chronic disease setting where current therapies are already highly effective and increasingly less burdensome.

Exhibit 11: Key gene therapy pipeline in GA/wet AMD

Asset	Company	Indication	Mechanism / Payload	Vector / Delivery	Stage	Trial
ABBV-RGX-314	AbbVie / REGENXBIO	Wet AMD	Ranibizumab-like anti-VEGF antibody fragment	AAV8; subretinal / suprachoroidal	Phase 3	Phase 2/3 ATMOSPHERE in wet AMD (primary completion Dec 2026) Phase 3 ASCENT in nAMD (primary completion Oct 2026) Phase 2 AAVIATE in nAMD (primary completion Aug 2026) Phase 2 ALTITUDE in diabetic retinopathy (primary completion June 2026)
4D-150	4D Molecular Therapeutics	Wet AMD	Aflibercept + anti-VEGF-C RNAi	Intravitreal AAV	Phase 3	Phase 3 4FRONT-1/2 in wet AMD (primary completion Nov 2028)
Ixo-vec (ADVM-022)	Adverum Biotechnologies	Wet AMD	Aflibercept expression	AAV7m8 intravitreal	Phase 3	Phase 3 ARTEMIS in nAMD (primary completion Dec 2026)
OCU410	Ocugen	Geographic atrophy (dry AMD)	RORA modifier gene therapy targeting multiple pathways	AAV5	Phase 2	Phase 1/2 AtMaDa in GA (primary completion Feb 2026)
LX102	Innoster Biotherapeutics	Wet AMD	Anti-VEGF gene therapy	AAV	Phase 3	Phase 3 STELLAR in wet AMD (primary completion Mar 2028)

Source: Company data, Data compiled by Goldman Sachs Global Investment Research

In DM1, the competitive landscape is fast-evolving, more complex, and highlights both the opportunity and risk of gene therapy. Current approaches in development are largely based on siRNA and antisense oligonucleotides (ASOs), which aim to reduce toxic

RNA transcripts or modulate splicing abnormalities. These modalities benefit from established delivery platforms and more controllable dosing, but typically require chronic administration and have shown variable efficacy depending on tissue penetration, particularly in muscle and CNS compartments. In contrast, Sanofi's AAV-based gene therapy (SAR446268) is designed to address the disease more upstream, via a one-time intervention targeting the underlying genetic defect, with the potential for sustained benefit across multiple tissues.

While these assets are not expected to be near-term contributors, they could become more strategically relevant under evolving R&D priorities. In that context, success in even one of these programs, especially in large indications such as AMD or high unmet need areas like DM1, could not only unlock incremental commercial opportunity but also enhance confidence in Sanofi's innovation capabilities, potentially positioning gene therapy as a more central pillar of the portfolio over time.

What might the capital allocation policy (particularly BD M&A) look like under the new CEO?

We have analysed Sanofi's historical M&A transactions (>US\$1bn) and estimated the ROIC for each acquisition over our forecast horizon, comparing these returns against the company's estimated 8% WACC to assess shareholder value creation. Our analysis suggests that outcomes vary materially by transaction type, with commercial deals generally offering the fastest path to value accretion, clinical-stage bolt-ons generating the strongest long-term return potential (albeit with higher pipeline risk), and platform acquisitions requiring greater upfront investment and longer timelines, while offering meaningful long-term ROIC expansion where multiple assets are successfully commercialised.

Exhibit 12: Summary of deals and value creation

Company	Deal value (\$bn)	Year of acquisition	Focus	Type	ROIC > WACC (GSe)
Genzyme	20.1	2011	Rare Diseases (Lysosomal Storage Disorders)	Commercial transformative	2014
Bioverativ	11.6	2018	Rare Blood Disorders (Hemophilia)	Commercial	2024
Ablynx	4.8	2018	Nanobody Technology Platform	Platform	2030
Synthorx ²	2.5	2019	Immuno-oncology (IL-2)	Clinical bolt-on	-
Principia Biopharma	3.7	2020	BTK Inhibitors	Clinical bolt-on	2027
Translate Bio	3.2	2021	mRNA Technology Platform/Lipid nanoparticle delivery	Platform	2029
Kadmon Holdings	1.9	2021	Rezurock, PD-1/IL-15 (KD050)	Commercial bolt-on	2024
Kymab Ltd	1.45	2021	OX40L (amlitelimab)	Clinical bolt-on	2030
Provention Bio ¹	2.9	2023	Type 1 Diabetes (Tzield)	Commercial bolt-on	-
Inhibrx	2.2	2024	Rare Disease (Alpha-1 Antitrypsin Deficiency)	Clinical bolt-on	2028
Blueprint	9.1	2025	Rare Immunology (Systemic Mastocytosis)	Commercial	2029
Dynavax	2.2	2025	Hepatitis-B vaccine	Commercial bolt-on	2028
Vicebio ³	1.6	2025	Molecular clamp platform	Platform	-
Vigil Neuroscience ⁴	1.05	2025	TREM-2 agonist Alzheimer's disease	Clinical bolt-on	-

1. Does not achieve ROIC>WACC

2. IL-2 asset discontinued and impairment charge recorded in 2022

3. SP0340 (VXB-251) and SP0341 (VXB-351) in Phase 1

4. SAR448851 (VG-3927) is in Phase 1

Source: Company data, Goldman Sachs Global Investment Research

Key takeaways from our analysis:

- **Commercial Deals (Transformative, Bolt-On & Other Commercial):** Commercial

transactions represent the broadest category within Sanofi's M&A history, with outcomes varying materially by deal size, asset maturity, and acquisition valuation. Genzyme (US\$20.1bn, 2011), the sole transformative commercial deal in our analysis, exceeded the 8% WACC by Year 3 on our estimates, and reached peak ROIC of ~19%, before moderating toward ~11% as key products matured. Among commercial bolt-ons, returns appear closely linked to valuation discipline and commercial execution. Kadmon (US\$1.9bn, 2021) and Dynavax (US\$2.2bn, 2025) exceeded WACC within three years on our estimates, generating peak ROIC of ~36% and ~19%, respectively. By contrast, Provention Bio (US\$2.9bn, 2023) does not exceed WACC in our model horizon, with peak ROIC of ~6%, highlighting the sensitivity of returns to peak sales assumptions relative to acquisition cost. Larger transactions such as Bioverativ (US\$11.6bn, 2018) and Blueprint (US\$9.1bn, 2025) fall between these outcomes, exceeding WACC by Year 6 and Year 4 respectively, with peak ROIC of ~23% and ~15%. **Overall, commercial deals appear to provide the clearest path to near-term value accretion, although ultimate returns remain dependent on acquisition valuation and asset durability.**

- **Clinical Bolt-On Deals:** Clinical-stage bolt-ons, in our view, offer some of the strongest long-term return profiles within Sanofi's M&A portfolio, albeit with longer timelines and higher pipeline risk. Principia Biopharma (US\$3.7bn, 2020) is projected to deliver peak ROIC of ~70% by 2037 on our estimates, supported by rilzabrutinib and tolebrutinib peak sales potential. Kymab (US\$1.45bn, 2021) follows with projected peak ROIC of ~64%, while Inhibrx (US\$2.2bn, 2024) reaches ~36% on our estimates. However, time-to-value creation is longer, with ROIC expected to exceed WACC in Year 4 for Inhibrx, Year 7 for Principia, and Year 9 for Kymab. The category also carries meaningful binary risk, as illustrated by Synthorx (US\$2.5bn, 2019), which resulted in a full impairment following discontinuation of its lead programme. **Overall, clinical bolt-ons in the US\$1–4bn range appear capable of generating attractive long-term returns where Sanofi can leverage existing therapeutic and commercial infrastructure.**
- **Platform Deals:** Platform acquisitions typically involve longer development timelines and upfront investment, with returns weighted toward outer years as multiple assets are developed from the underlying technology. Ablynx (US\$4.8bn, 2018) is the clearest example in our analysis, with the nanobody platform expected to exceed WACC by Year 12 and peak ROIC reaching ~48% by 2037 on our estimates, supported by multiple pipeline assets including Cablivi, brivekimig and lunsekimig. Translate Bio (US\$3.2bn, 2021) follows a similar pattern, with ROIC expected to exceed WACC by Year 8 and peak ROIC reaching ~21%, reflecting the longer development timelines associated with mRNA platforms. Vicebio (US\$1.6bn, 2025) remains too early stage for meaningful returns analysis. **Overall, platform transactions appear less suited to near-term value accretion given the upfront investment required and longer time to commercialisation, but can support significant long-term ROIC expansion where the platform generates multiple commercially successful assets.**

What could Sanofi Do? Our view on M&A strategy ahead of Dupixent LoE

In terms of capital allocation and M&A, Sanofi's approach has been focused on investing in organic growth, pursuing selective bolt-on acquisitions, maintaining a progressive dividend and executing share buybacks. On the 1Q26 earnings call, the company noted that it would generally target bolt-on acquisitions in the €2-5bn range, while emphasising that it would not discount larger opportunities if they are attractive and offer shareholder value. At our Innovation Summit in March 2026, Sanofi indicated that it expects increased activity in immunology, continued but more selective investment in rare diseases, right-sizing in vaccines, and sees ophthalmology as a potential area of breakout growth. Further, on platforms management noted a focused approach, prioritizing areas of strength such as monoclonal and bispecific antibodies, where more assets are expected to enter the clinic and highlighted emerging capabilities in blood brain barrier shuttle and AAV platforms. **We see Sanofi having balance sheet capacity of c.US\$39bn for M&A, if we were to assume that the company was willing to move to 3x ND/EBITDA (3x being a typical limit historically for Pharma companies for financial leverage), vs. c.0.6x currently (GSe 2026E) and an estimated c.0.2x by end-2027E (GSe). Of note, the company has indicated that it could invest up to €15bn in 2026 in BD M&A, while retaining an AA credit rating.**

Given the approaching loss of exclusivity for Dupixent, our ROIC analysis suggests Sanofi may be better positioned pursuing a balanced M&A strategy combining near-term commercial bolt-ons with selective clinical-stage acquisitions, rather than relying on a single large-scale transformative transaction. Commercial bolt-ons appear to offer the fastest path to value accretion, typically exceeding WACC within ~3 years on our estimates, while also providing a nearer-term revenue contribution. However, our analysis indicates that returns remain highly sensitive to acquisition valuation. Clinical-stage bolt-ons in the sub-US\$5bn range appear to offer the strongest long-term return potential, with projected peak ROIC ranging from ~36%-70% on our estimates, particularly where Sanofi can leverage existing capabilities in immunology, neurology, and rare disease. By contrast, while large transformative acquisitions can provide immediate scale, our analysis suggests they may carry more moderate long-term returns alongside greater integration and portfolio maturity risk, as illustrated by the declining ROIC profile observed in Genzyme over time.

Platform acquisitions can also generate substantial long-term ROIC expansion, although the return profile is typically back-end weighted and requires longer development timelines and significant upfront investment. Given the timing of Dupixent's anticipated LoE, we believe Sanofi is likely to prioritise transactions with clearer medium-term revenue contribution. **Overall, our analysis supports a more diversified acquisition approach, combining selective clinical-stage bolt-ons with commercially accretive assets, while maintaining disciplined acquisition valuation.**

Other factors to consider

- **Sanofi retains an economic interest in the transthyretin (TTR) space through its RNAi therapeutics alliance with Alnylam Pharmaceuticals, under which it is entitled to royalties on Amvuttra, while Alnylam retains global development and commercialization control and is therefore exposed to evolving competitive dynamics that could impact Amvuttra and associated royalty streams.** Amvuttra is currently well positioned as a leading RNAi-based therapy in ATTR cardiomyopathy, supported by c.80%-90% TTR knockdown, a convenient quarterly dosing profile, and increasing clinical traction as the market shifts beyond stabilizers. Data expected in 2H26 from the Phase 3 CARDIO-TTRansform study of Wainua (AstraZeneca / Ionis Pharmaceuticals), which uses a highly relevant primary composite endpoint of cardiovascular mortality and recurrent CV events and allows background use of stabilizers such as tafamidis, could influence competitive dynamics, particularly if positive in validating antisense and enabling a combination paradigm not currently reflected in the Amvuttra label. The recent update on trial design and baseline characteristics were presented at the ESC Heart Failure Congress and point to a robust dataset in 2H26, while we note a high bar for success (see our detailed note [here](#)). Looking further ahead, nucsiran represents Alnylam's next-generation TTR silencer with more durable knockdown and potential for infrequent dosing, and while a KOL at our recent Innovation summit described the asset as potentially "game changing", it sits outside the Sanofi collaboration and any future transition toward nucsiran could result in a displacement of Amvuttra and a corresponding erosion of these royalties over time, a headwind to Sanofi's Other operating income line.

Exhibit 13: Potential impact of nucsiran on Amvuttra royalties

	YoY displacement after nucsiran launch in 2030					
	20%			40%		
	Amvuttra sales	Royalty rate	Royalties	Amvuttra sales	Royalty rate	Royalties
2030 GSe Amvuttra sales: \$8132mn						
2031	6505.6	28%	1,840	4879.2	28%	1,353
2032	5,204	28%	1,450	2,928	26%	767
2033	4,164	27%	1,138	1,757	24%	416
2034	3,331	27%	888	1,054	10%	102
2035	2,665	26%	688	632	19%	122
2036	2,132	25%	528	379	19%	73
2037	1,705	23%	400	228	15%	35

Source: Goldman Sachs Global Investment Research

Forecast changes

Key updates to our estimates include: (i) FX updates to sales and Business EPS (moving from -2.4% to -1.3% impact on sales and from -3.5% to -1.9% on Business EPS); (ii) lowering of riliprubart PoS from 40% to 10% following the recent update on Phase 3 MOBILIZE study discontinuation; (iii) increased tolebrutinib PoS to 100% from 75% following EU approval in nrSPMS; (iv) incorporating the impact of the €2.3bn bond issue; and (v) we now model biosimilar entry/price pressure for Dupixent starting from 2033 and reduce our terminal growth rate from 0% to -2% given R&D disappointments, implying a -4% impact to our DCF estimates.

Exhibit 14: Forecast changes

	2026	2027	2028	2029	2030
Sales - New	46,657	50,015	53,127	56,037	58,453
Sales - Old	46,509	49,836	52,971	55,885	58,327
Diff %	0.3%	0.4%	0.3%	0.3%	0.2%
Diff Abs	148	179	156	152	126
Operating profit - New	13,024	14,494	15,677	17,084	17,975
Operating profit - Old	13,129	14,163	15,573	16,987	17,910
Diff %	-0.8%	2.3%	0.7%	0.6%	0.4%
Diff Abs	-105	331	104	98	66
EPS - New	8.46	9.50	10.45	11.56	12.43
EPS - Old	8.49	9.23	10.17	11.10	11.77
Diff %	-0.4%	3.0%	2.8%	4.2%	5.6%
Diff Abs	-0.03	0.27	0.28	0.47	0.66

Source: Goldman Sachs Global Investment Research

Valuation

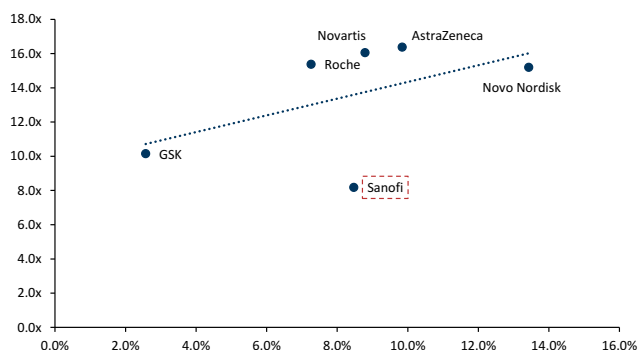
Sanofi trades at 8.2x 2027E PE, a c.35% discount to EU Biopharma peers, and on a PEG (based on 2027-30E EPS CAGR) of c.1.0x versus a peer average of 1.8x. In our view, the current valuation discount reflects the market balancing strong near-term earnings delivery against uncertainty over the sustainability of growth beyond Dupixent. While the company continues to deliver robust earnings growth, supported by Dupixent, there remains limited visibility on the next generation of growth drivers, with R&D execution yet to produce the level of late-stage pipeline successes required to materially shift investor expectations.

The company is entering a period of tougher comparators following strong execution through 1Q26, with management cautioning on the outlook for the remainder of the year. Delivery of double-digit EPS growth is still a strong outcome, although, given that we are in line with consensus for FY26, this appears to be reflected in expectations, limiting the scope for continued earnings momentum. In addition, visibility on Dupixent patent extension is likely to remain limited until a challenge emerges, and expected pipeline news flow is insufficient to materially improve investor confidence, in our view. As a result, despite Sanofi's apparent valuation discount, we see limited potential for the valuation gap to narrow in the near term. We believe the most likely catalyst through 2026 is a strategic update from the new CEO. However, any improvement in sentiment is likely to require not only a credible strategic roadmap but also proof of successful execution.

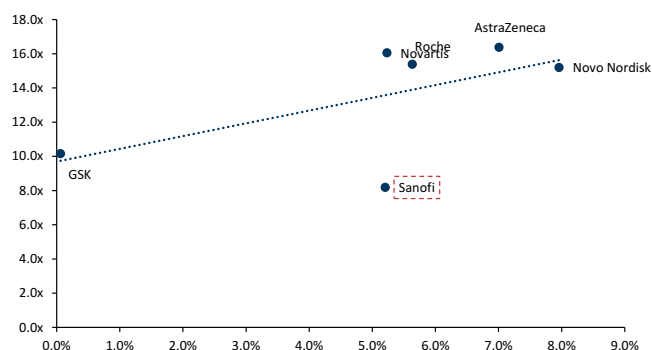
Exhibit 15: EU BioPharma Valuation summary

	Market cap outstanding (\$bn)	Sales growth (USD)						EPS growth (USD)						P/E trading multiples				2027 PEG	
		2026	2027	2028	2029	2030	27-30 CAGR	2026	2027	2028	2029	2030	27-30 CAGR	2026	2027	2028	2029	2027-30 EPS CAGR	2027-30 Sales CAGR
AstraZeneca	293,378	7.3%	6.3%	7.9%	6.5%	6.6%	7.0%	10.7%	14.0%	11.6%	9.3%	8.6%	9.8%	18.7x	16.4x	14.7x	13.4x	1.7x	2.3x
Bayer	51,653	2.5%	0.9%	3.5%	4.3%	4.3%	4.1%	-9.8%	4.6%	17.8%	18.5%	12.0%	16.1%	10.5x	10.1x	8.6x	7.2x	0.6x	2.5x
GSK	107,058	5.7%	2.7%	1.1%	1.1%	-2.0%	0.1%	7.3%	6.1%	3.2%	3.9%	0.6%	2.6%	10.8x	10.2x	9.8x	9.5x	4.0x	170.8x
Merck KGaA	72,706	4.0%	2.9%	6.5%	5.4%	5.3%	5.7%	-0.8%	3.6%	14.3%	10.0%	12.6%	12.3%	17.9x	17.3x	15.1x	13.8x	1.4x	3.0x
Novartis	297,330	3.1%	6.2%	6.3%	6.9%	2.6%	5.2%	-2.1%	10.6%	11.5%	10.5%	4.5%	8.8%	17.8x	16.1x	14.4x	13.0x	1.8x	3.1x
Novo Nordisk	214,342	5.7%	-7.3%	5.2%	7.1%	11.6%	8.0%	11.6%	-18.7%	6.1%	12.3%	22.4%	13.4%	12.4x	15.2x	14.3x	12.8x	1.1x	1.9x
Roche	337,606	7.8%	2.2%	5.1%	6.9%	4.9%	5.6%	8.9%	5.8%	6.1%	10.0%	5.6%	7.3%	16.3x	15.4x	14.5x	13.2x	2.1x	2.7x
Sanofi	104,223	9.6%	4.6%	6.1%	5.3%	4.3%	5.2%	12.1%	6.4%	10.2%	9.1%	6.1%	8.5%	8.7x	8.2x	7.4x	6.8x	1.0x	1.6x
Europe Biopharma sector	1,478,296	6.0%	2.5%	5.3%	5.6%	4.8%	4.5%	5.4%	3.3%	8.7%	9.7%	8.0%	7.2%	14.6x	14.1x	12.9x	11.7x	1.9x	3.1x
Sector ex. NVO	1,263,954	6.0%	3.7%	5.3%	5.5%	3.9%	4.8%	4.5%	8.0%	9.4%	9.7%	6.3%	9.0%	15.0x	13.9x	12.7x	11.5x	1.5x	2.9x

Source: Factset, Goldman Sachs Global Investment Research

Exhibit 16: 2027E P/E vs. 2027-30E EPS CAGR: Sanofi trades at 1.0x PEG, a c.46% discount to peers

Source: Factset, Goldman Sachs Global Investment Research

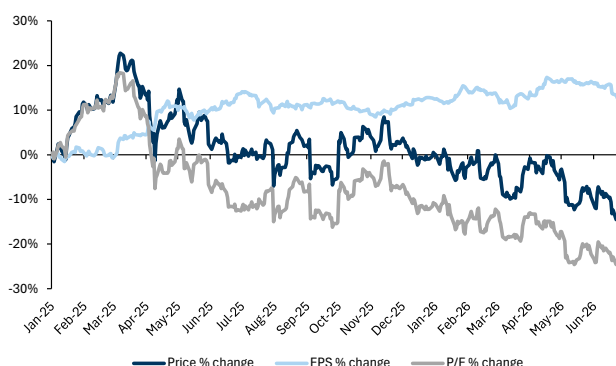
Exhibit 17: 2027E P/E vs. 2027-30E Sales CAGR: Sanofi trades at 1.6x PEG, a c.43% discount to peers

Source: Factset, Goldman Sachs Global Investment Research

Sanofi has materially underperformed the SXDP since January 2025, with the shares declining c.11% versus a c.17% gain for the index. The relative underperformance began to emerge in June 2025 (as seen in [Exhibit 18](#)) and has accelerated since year-end 2025, with the stock reaching a five-year low of €72.72 on 13 May 2026. This represents a significant decline from the all-time high of €119.16 reached in March 2025. In our view, the weakness has been driven primarily by a series of pipeline setbacks that have weighed on investor confidence in the company's R&D delivery. Key disappointments include mixed Phase III results for itepekimab (shares -5% on the day), lower-than-expected efficacy data for amlitelimab (shares -8% on the day), and the Phase III failure and subsequent Complete Response Letter (CRL) for tolebrutinib in PPMS (shares -3% on the day). Despite a solid underlying growth profile, with the company expected to deliver above-peer sales and EPS CAGR over 2027–2030, supported primarily by continued momentum from Dupixent, sentiment remains measured. This can be seen in [Exhibit 19](#), where the PE multiple de-rating is the key driver of the share price (12M forward EPS upgrades being c.+13% and 12M forward PE multiple downgrades being c.-21%). **Overall, we believe the market is currently discounting this earnings growth potential due to a lack of confidence on Sanofi's innovation engine and the lack of near-term best-in-class/first-in-class pipeline catalysts.**

Exhibit 18: Sanofi performance vs. SXDP since Jan 2025

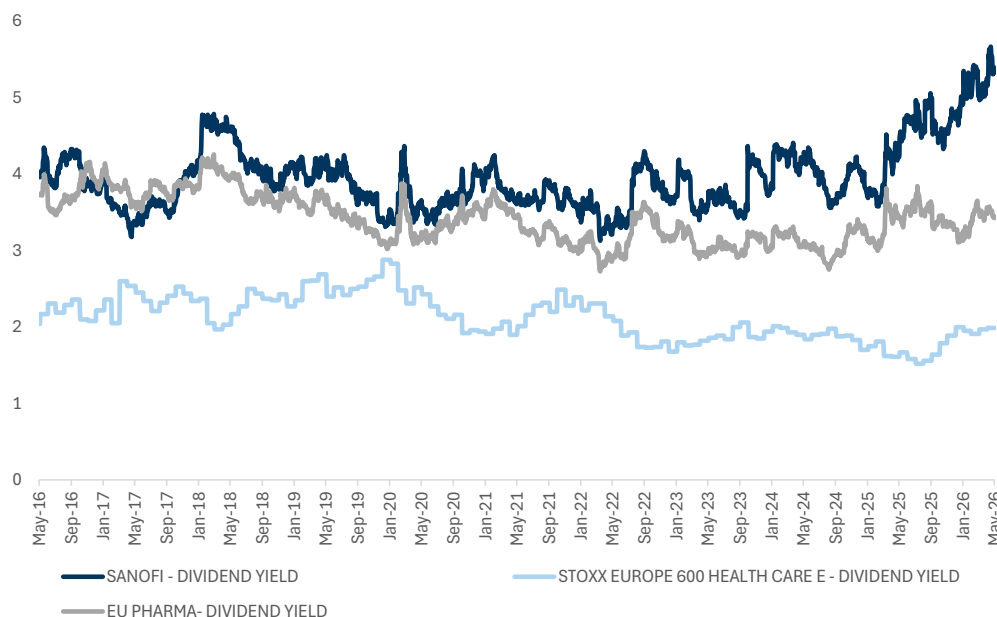
Source: LSEG Data & Analytics

Exhibit 19: Sanofi performance: share price, 12m fwd EPS and 12m fwd PE

Source: LSEG Data & Analytics

Currently, Sanofi's dividend yield is c.5.5%, compared to a sector average of c.3.5%. Historically, Sanofi's dividend yield has been slightly above the sector average, with a 10-year average of c.4.0% versus c.3.4% for peers, reflecting the company's progressive dividend policy. However, the gap has begun to widen from May 2025, following the share price decline from its March 2025 peak. While continued dividend growth has contributed to the increase, the majority of the expansion in yield has coincided with the decline in the share price. **Overall, while the elevated yield, together with ongoing share buybacks, increasingly positions Sanofi as a "value stock", total shareholder returns remain constrained by recent share price performance, with income returns only partially offsetting weaker capital returns.**

Looking ahead, a more meaningful improvement in shareholder returns is likely to require either greater confidence in the company's ability to deliver on its internal pipeline or successful execution of external innovation through business development (BD) and M&A. **In our view, BD/M&A remains an important component of a supportive capital allocation framework, complementing the existing dividend and share buyback policy.**

Exhibit 20: Sanofi vs. SXDP and EU Pharma dividend yield

Source: LSEG Data & Analytics

Price target and key risks

We are Neutral-rated. We derive our EU large cap pharma price targets using a 50:50 blend of a DCF-based valuation and a P/E multiple-based approach. Our bottom-up DCF methodology utilises an 8% WACC and -2% TGR (previously 0% TGR, updated to reflect recent R&D disappointments) and suggests a valuation of €83 per share (previously €87 per share). Our P/E methodology utilises an 8.5x target multiple applied to 2027E EPS, and suggests a valuation of €85 per share (previously €92 per share). Blended, this results in a 12-month price target of €84 (previously €89) and ADR price target of \$49 (previously \$51).

Upside risks to our view and price target:

1. Strong clinical pipeline readouts, including Phase 2 readouts, driving a positive reaction;
2. Faster-than-expected sales growth of Dupixent, Altuviiio, Ayvakit and Beyfortus, driving positive earnings momentum; and,
3. Capital allocation (i.e. extending the current buyback programme or M&A) could also have a positive impact.

Downside risks:

1. Disappointments on clinical pipeline readouts driving concerns over the long-term replacement strategy for Dupixent;
2. Downward EPS consensus revisions due to FX in the short term but also longer term in other operating income as Regeneron payaway is factored in; and,

3. Slower-than-expected sales growth of Dupixent, Altuviiiio and Beyfortus.

Disclosure Appendix

Reg AC

We, James Quigley, Rajan Sharma, Shyam Kotadia, Kaaviya Ganesan, Max Da, Ph.D. and Theodora Rowe Beadle, hereby certify that all of the views expressed in this report accurately reflect our personal views about the subject company or companies and its or their securities. We also certify that no part of our compensation was, is or will be, directly or indirectly, related to the specific recommendations or views expressed in this report.

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Contributing Authors: James Quigley Goldman Sachs International, Rajan Sharma Goldman Sachs International, Shyam Kotadia Goldman Sachs International, Kaaviya Ganesan Goldman Sachs India SPL, Max Da, Ph.D. Goldman Sachs International, Theodora Rowe Beadle Goldman Sachs International.

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Growth is based on a stock's forward-looking sales growth, EBITDA growth and EPS growth (for financial stocks, only EPS and sales growth), with a higher percentile indicating a higher growth company. **Financial Returns** is based on a stock's forward-looking ROE, ROCE and CROCI (for financial stocks, only ROE), with a higher percentile indicating a company with higher financial returns. **Multiple** is based on a stock's forward-looking P/E, P/B, price/dividend (P/D), EV/EBITDA, EV/FCF and EV/Debt Adjusted Cash Flow (DACF) (for financial stocks, only P/E, P/B and P/D), with a higher percentile indicating a stock trading at a higher multiple. The **Integrated** percentile is calculated as the average of the Growth percentile, Financial Returns percentile and (100% - Multiple percentile).

Financial Returns and Multiple use the Goldman Sachs analyst forecasts at the fiscal year-end at least three quarters in the future. Growth uses inputs for the fiscal year at least seven quarters in the future compared with the year at least three quarters in the future (on a per-share basis for all metrics).

For a more detailed description of how we calculate the GS Factor Profile, please contact your GS representative.

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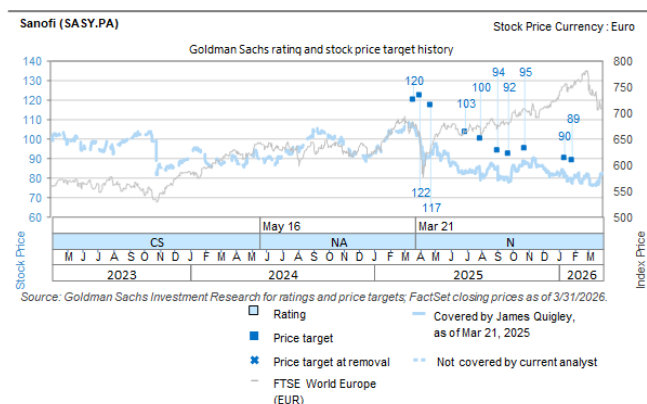
Goldman Sachs Investment Research global Equity coverage universe

	Rating Distribution				Investment Banking Relationships		
	Buy	Hold	Sell		Buy	Hold	Sell
Global	50%	34%	16%		65%	60%	45%

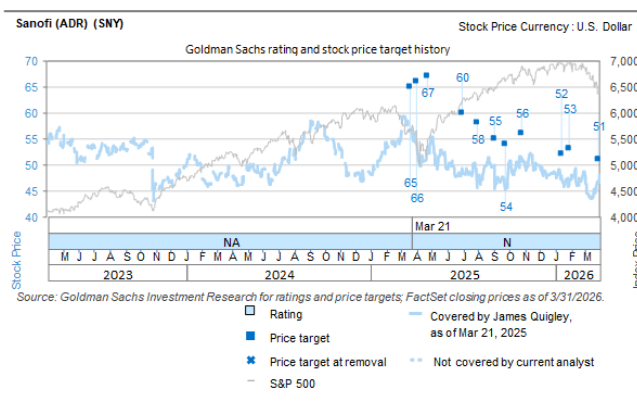
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Relationships chart reflects the percentage of subject companies within each rating category for whom Goldman Sachs has provided investment banking services within the previous twelve months.

Price target and rating history chart(s)



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The price targets shown should be considered in the context of all prior published Goldman Sachs research, which may or may not have included price targets, as well as developments relating to the company, its industry and financial markets.

Target price history table(s)

Sanofi (ADR) (SNY)

Date of report	Target price (\$)	Closing price (\$)
30-Mar-26	51.00	46.72
30-Jan-26	53.00	47.04
15-Jan-26	52.00	47.47
27-Oct-25	56.00	51.41
25-Sep-25	54.00	45.07
04-Sep-25	55.00	45.33
01-Aug-25	58.00	46.75
02-Jul-25	60.00	49.31
24-Apr-25	67.00	53.54
02-Apr-25	66.00	53.95
21-Mar-25	65.00	56.90

Sanofi (SASY.PA)

Date of report	Target price (€)	Closing price (€)
30-Jan-26	89.00	79.20
15-Jan-26	90.00	81.63
27-Oct-25	95.00	88.13
25-Sep-25	92.00	77.52
04-Sep-25	94.00	78.96
01-Aug-25	100.00	80.34
02-Jul-25	103.00	83.61
24-Apr-25	117.00	93.35
02-Apr-25	122.00	100.40
21-Mar-25	120.00	105.84

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